

EVIDENCE / DEVELOPMENT / QUALITY / COMMUNICATION

Pharmaceutical English

State the evidence. Qualify the claim.
Close the communication gap.

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Eight scenarios. Eight extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
160 dialogue turns / 80 numbered dialogue gaps
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For pharmaceutical development, clinical, regulatory, safety, quality, medical, and access teams.

2026 edition

Eight scenarios. Eight skills.

Follow eight cross-functional conversations from early evidence to development decisions, safety, quality, and access.

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A laboratory signal becomes a patient-benefit claim | Replace an overstated development claim with a precise finding and a testable next question.

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A favorable secondary result takes over the headline | Explain a trial readout without replacing the prespecified question or overstating statistical certainty.

05 Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates 40

An incomplete safety report | Preserve reported facts, avoid premature causality judgments, and route a safety report without waiting for perfect information.

06 CMC, CGMP, Quality Events, Manufacturing, and Supply 49

A gap in the batch record | Report a quality-documentation gap precisely and separate investigation status from batch disposition.

07 Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries 58

Stronger wording changes the claim | Identify an unsupported promotional implication and return a precisely scoped claim through the required review route.

08 Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management 67

Two sources, different questions | Explain differences between study designs and keep evidence, economic assumptions, and access decisions distinct.

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Precision protects the meaning.

Pharmaceutical conversations often connect specialists who use similar words for different purposes. A result, a hypothesis, a submission, and an approval are not interchangeable. These fictional cases practice concise clarification, careful evidence language, and explicit handoffs without turning English exercises into technical operating instructions.

Name the evidence level

Say whether a finding comes from a laboratory model, a clinical trial, or routine-care data. Keep the population, comparator, outcome, and limitations attached to the claim.

"The laboratory finding supports a hypothesis; it does not establish patient benefit."

Verify the controlling document

A familiar attachment may be outdated or not applicable to a particular site. Ask for the document identifier, version, approval status, and effective status before treating conflicting wording as settled.

"Please confirm the version authorized for this site and the exact section that governs the visit."

Report uncertainty without losing urgency

Incomplete information is not the same as evidence that nothing happened. Follow the applicable safety or quality process and preserve what was reported, what is missing, and who owns follow-up.

"The report is incomplete; I am routing the available information through the required channel now."

Keep review and approval distinct

Receiving a submission, reviewing a draft, and authorizing use are different events. State the actual status and next decision instead of using reassuring language that implies more than the record supports.

"Comments are resolved, but final approval for this version is still pending."

PROFESSIONAL BOUNDARIES

Original fictional language practice, not medical advice, clinical training, regulatory guidance, or authorization to perform regulated work. US terminology is used where identified; requirements vary by jurisdiction, product, and context. Follow current approved procedures, qualified professional judgment, and applicable requirements.

A laboratory signal becomes a patient-benefit claim

A Read the situation

Replace an overstated development claim with a precise finding and a testable next question.

An internal slide for fictional candidate PX-17 says it improves patient mobility. The available experiment used a cell-based assay, not patients. Across three experimental runs, the team observed a change in a disease-related marker at one tested concentration. Human exposure, tolerability, and clinical benefit have not been established. Scientist Leila and development lead Owen are preparing a portfolio discussion. The proposed target product profile describes a desired future benefit, not an achieved product characteristic. No clinical study is authorized by this discussion.

PEOPLE / Leila: Translational scientist | Owen: Development lead

A NARROWER CLAIM CAN BE MORE USEFUL

Do not soften an unsupported assertion merely by adding promising. State what was measured, where it was measured, and what remains unknown. Then connect the result to the next question. This lets a colleague support further investigation without endorsing a conclusion the evidence cannot carry.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Where was the observed result obtained?

- | | |
|---|--|
| A In a cell-based assay across three experimental runs | B In a completed patient mobility trial |
| C In routine treatment at multiple hospitals | D In an approved product label |

2. What does the target product profile represent here?

- | | |
|--|---|
| A Desired future characteristics to guide development | B Proof that patient mobility has improved |
| C Authorization to begin a clinical study | D A final regulatory approval |

3. Which claim exceeds the evidence?

- | | |
|---|--|
| A A disease-related marker changed in the assay. | B Human tolerability remains unestablished. |
| C Further evidence is required. | D PX-17 improves patient mobility. |

Answers and explanations: [page 76](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

unmet medical need

A health-related need not adequately addressed by available options.

IN USE characterize the unmet medical need

target product profile

A planning description of desired future product characteristics.

IN USE refine the target product profile

development candidate

A selected investigational substance being evaluated for further development.

IN USE nominate a development candidate

mechanism of action

The biological process through which an intervention produces an effect.

IN USE investigate the mechanism of action

target engagement

Evidence that an intervention interacts with its intended biological target.

IN USE assess target engagement

proof of concept

Evidence supporting the feasibility of a defined scientific or clinical idea.

IN USE specify the proof-of-concept question

cell-based assay

A laboratory test using cells to assess a specified response.

IN USE qualify the cell-based assay

biomarker

A measured characteristic indicating a biological process or response.

IN USE evaluate a candidate biomarker

surrogate endpoint

A substitute outcome used to predict a clinical outcome, with context-dependent support.

IN USE justify a surrogate endpoint

clinical benefit

A meaningful favorable effect on how patients feel, function, or survive.

IN USE establish clinical benefit

translational evidence

Information connecting findings across experimental and human contexts.

IN USE strengthen translational evidence

biological plausibility

A reasoned basis for believing a biological explanation could be credible.

IN USE assess biological plausibility

Precision check

A biomarker change is not automatically clinical benefit. Its meaning depends on the marker, context, and supporting evidence. Here, one concentration and three laboratory runs do not establish human exposure, tolerability, dose response, or improved mobility.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

dose-response relationship

How a measured effect varies with administered dose.

IN USE characterize the dose-response relationship

exposure-response relationship

How an effect varies with drug exposure in the body.

IN USE examine exposure-response relationships

pharmacokinetics

Study of the time course of drug exposure through absorption, distribution, metabolism, and elimination.

IN USE characterize pharmacokinetics

pharmacodynamics

Study of a drug's biological effects and their relation to exposure.

IN USE measure pharmacodynamic effects

nonclinical study

Research conducted outside human clinical studies.

IN USE interpret nonclinical findings

tolerability

The extent to which unwanted effects can be endured in a defined context.

IN USE evaluate tolerability

therapeutic window

The exposure range associated with desired effects and acceptable toxicity in context.

IN USE investigate the therapeutic window

reproducibility

The ability to obtain consistent findings under specified repeat conditions.

IN USE test reproducibility

assay limitation

A restriction affecting what a test can establish.

IN USE disclose assay limitations

go/no-go criterion

A defined condition informing whether development proceeds or stops.

IN USE set go/no-go criteria

evidence gap

Missing information needed to support a proposed conclusion.

IN USE prioritize evidence gaps

development milestone

A defined point of progress or review in a development program.

IN USE plan the next development milestone

Precision check

Target product profile is often shortened to TPP; mechanism of action to MOA; pharmacokinetics to PK; and pharmacodynamics to PD. Expand the terms when needed, and distinguish a planned characteristic from an observed result. A development goal must not silently become an evidence claim.

Phrases you can actually use

C Notice the language / 1 of 2

LOCATE THE EVIDENCE

"The observation comes from a cell-based assay, not a patient study."

PROTECT THE DISTINCTION

"Clinical benefit has not been established."

ASK FOR THE BRIDGE

"What evidence connects this marker to the intended patient outcome?"

NAME THE LIMITATION

"We tested one concentration, so this does not characterize a dose-response relationship."

NARROW THE CLAIM

"We observed a marker change under the tested conditions."

CLARIFY THE ASPIRATION

"The target product profile describes what we aim to demonstrate."

QUALIFY REPETITION

"Three runs support repeatability within these conditions, not general clinical effectiveness."

STATE THE NEXT QUESTION

"We need to test whether the finding persists under relevant follow-up conditions."

Observed

Use for the measured result, not for an extrapolated patient benefit.

Suggests

State exactly what the result suggests and why.

Proof of concept

Specify the concept; laboratory and clinical proof are different claims.

Phrases you can actually use

C Notice the language / 2 of 2

AVOID IMPLIED SAFETY

"The current experiment does not establish human tolerability."

SEPARATE STAGES

"An early signal supports investigation, not a treatment recommendation."

CLARIFY THE MEASURE

"Are we discussing target engagement, a biomarker, or a patient outcome?"

MARK AUTHORITY

"This portfolio discussion does not authorize a clinical study."

CHECK THE POPULATION

"Which patients would the proposed product eventually be intended to serve?"

REQUEST A CRITERION

"What result would support the next development decision?"

KEEP THE GOAL CONDITIONAL

"The desired benefit remains a hypothesis to be evaluated."

CLOSE ACCURATELY

"Retain the finding, its conditions, and the unresolved evidence gap."

TPP

This is a planning tool, not automatically an approved label.

At one concentration

This qualifier limits what can be said about response across concentrations.

In patients

Do not add this phrase when the evidence comes only from laboratory work.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which replacement is best supported?

- A** PX-17 is safe at every human dose.
- B** PX-17 has an approved mobility indication.
- C** PX-17 changed the marker in the tested cell assay; patient benefit remains unestablished.
- D** PX-17 restores mobility in patients because the marker changed.

2. What does testing one concentration fail to characterize?

- A** How the effect changes across different concentrations
- B** Whether the tested assay recorded any response
- C** Whether the experiment used cells
- D** The number of experimental runs stated in the brief

3. Which question best tests the evidence bridge?

- A** How well does this marker predict the intended patient outcome in this context?
- B** Which adjective makes the slide sound most certain?
- C** Can we delete the laboratory setting to simplify the claim?
- D** Does a planning goal count as an observed clinical result?

4. What can the portfolio discussion decide on the supplied authority?

- A** Final marketing authorization
- B** A universal human dosing recommendation
- C** A proposed next investigation for appropriate review, not automatic clinical-study authorization
- D** Immediate patient treatment with the candidate

[Answers and explanations: page 76.](#)

A laboratory signal becomes a patient-benefit claim

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

target product profile / biomarker / evidence gap / cell-based assay / reproducibility / dose-response relationship / clinical benefit / go/no-go criterion / development milestone / tolerability

- 01 **Owen:** The portfolio slide says PX-17 improves patient mobility. I thought the latest package contained laboratory work only. Have I missed a clinical result?
- 02 **Leila:** No. The result comes from a ¹ _____. We saw the marker change across three runs at one tested concentration. Nobody measured mobility in patients.
- 03 **Owen:** Then the headline has moved beyond the evidence. The team wanted a clear link to the disease, but the wording makes that link sound demonstrated.
- 04 **Leila:** We can describe the ² _____ response precisely and explain why it interests us. We still need evidence connecting that response to the outcome we hope to influence.
- 05 **Owen:** The commercial planning page also lists improved mobility. That is in the target profile, so perhaps someone copied it into the result summary.
- 06 **Leila:** The ³ _____ is aspirational at this stage. We should label the desired outcome separately from the finding so readers do not confuse the development goal with achieved performance.
- 07 **Owen:** Can we call this proof of concept? The phrase is familiar, but different colleagues may hear very different claims when they read it.
- 08 **Leila:** Specify the concept and setting. It is not evidence of ⁴ _____. A broad proof-of-concept label could conceal exactly the distinction we are trying to restore.
- 09 **Owen:** We also tested only one concentration. The slide's rising arrow looks like an established relationship between dose and effect, even though we have no such series.
- 10 **Leila:** Remove that implication. A ⁵ _____ has not been characterized here. The visual should not make a stronger statement than the accompanying words or the experiment itself.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Owen:** What can we say about the three runs? I do not want to discard useful repetition merely because the result is still early.
- 12 **Leila:** Describe the observed consistency and the conditions. It informs
6 _____ within this setup, while leaving broader confirmation open. We should also report any relevant variation rather than showing only the most favorable run.
- 13 **Owen:** Someone has added well tolerated to the summary. There were no human participants, so I cannot see what that statement is based on.
- 14 **Leila:** Human 7 _____ is unestablished. We can report the actual observations, but we should not translate the absence of a particular laboratory finding into a general human safety claim.
- 15 **Owen:** Let us make the next decision concrete. What missing information matters most before the program can support a stronger development proposal?
- 16 **Leila:** Start with the 8 _____ between the assay response and the intended outcome. The scientific team should define the follow-up question and its limits through the appropriate development process.
- 17 **Owen:** The committee will ask what would count as enough progress to continue. A broad instruction to produce more data will not help them compare programs.
- 18 **Leila:** Propose a defined 9 _____ for qualified review. It should match the next decision, without pretending that an early laboratory threshold alone establishes a product suitable for patients.
- 19 **Owen:** I will separate the observation, the hypothesis, and the requested next step. We can keep the result interesting without making the conclusion premature.
- 20 **Leila:** Good. The next 10 _____ remains subject to the required review. This discussion does not authorize a clinical study, and the final slide should say no more than the evidence supports.

ANSWERS / EXPLANATIONS: PAGE 77

1. cell-based assay | 2. biomarker | 3. target product profile | 4. clinical benefit | 5. dose-response relationship | 6. reproducibility | 7. tolerability | 8. evidence gap | 9. go/no-go criterion | 10. development milestone

A goal is not a result

F Say it

A planning profile targets once-daily use. The candidate has no human dosing data. A laboratory test measured target engagement but did not measure patient symptoms.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unestablished / goal / improvement / target engagement

Scientist: "Once-daily use is currently a ¹ _____."

Reviewer: "The measured laboratory finding concerns ² _____."

Scientist: "Human dosing remains ³ _____."

Reviewer: "We cannot claim symptom ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 77

1. goal | 2. target engagement | 3. unestablished | 4. improvement

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A meeting question that asks for blanket agreement

A Read the situation

Frame a focused regulatory question and accurately record the scope of the response.

A US development team is preparing an agency meeting package for fictional investigational product RV-8. Its draft asks whether the entire development plan is acceptable. The immediate unresolved question concerns the proposed patient population and the rationale for a planned study endpoint. Supporting summaries exist, but two supporting reports are unfinished. Regulatory lead Priya and clinical lead Evan must distinguish the sponsor proposal, available evidence, and specific advice requested. No agency response has been received, and meeting preparation does not constitute authorization to begin the study.

PEOPLE / Priya: Regulatory lead | Evan: Clinical lead

MAKE THE QUESTION ANSWERABLE

A focused question gives reviewers a proposal, supporting rationale, and a specific point on which advice is sought. Avoid treating a useful discussion as blanket agreement with every part of a program. Meeting records should preserve conditions, unresolved issues, and any distinction between advice and formal authorization.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is wrong with the current question?

A It identifies too narrow a product name.

B It asks for information already supplied as final approval.

C It asks broadly about the entire plan instead of the specific unresolved proposal.

D It contains a confirmed agency decision.

2. What is the status of two supporting reports?

A Irrelevant because the meeting is scheduled

B Unfinished

C Approved by the agency

D Replaced by a marketing authorization

3. Which statement is accurate now?

A An NDA has been approved for RV-8.

B No agency response has been received.

C The study is authorized because a package is being prepared.

D The entire program has formal agreement.

Answers and explanations: page 78.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

regulatory strategy

A plan for addressing applicable regulatory requirements and interactions.

IN USE align the regulatory strategy

regulatory pathway

The applicable route for development, review, or authorization.

IN USE confirm the regulatory pathway

investigational new drug application

A US submission supporting proposed clinical investigation of a drug.

IN USE prepare an IND submission

new drug application

A US application seeking approval to market a new drug.

IN USE plan the NDA content

biologics license application

A US application seeking licensure of a biological product.

IN USE prepare the BLA strategy

agency interaction

A formal or informal exchange with a regulatory authority.

IN USE document the agency interaction

meeting package

Materials submitted to support a scheduled regulatory discussion.

IN USE assemble the meeting package

briefing document

A structured account of background, evidence, proposals, and questions.

IN USE finalize the briefing document

sponsor position

The development sponsor's proposed approach and rationale.

IN USE state the sponsor position

focused question

A question limited to a defined issue that can receive a useful answer.

IN USE formulate a focused question

supporting rationale

The reasoning and evidence behind a proposal.

IN USE substantiate the supporting rationale

information gap

Missing material relevant to assessing a proposal.

IN USE disclose an information gap

Precision check

IND means investigational new drug application; NDA means new drug application; BLA means biologics license application in US usage. These terms name different regulatory submissions. An IND is not a marketing approval, and preparing a meeting package does not authorize a study.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

submission readiness

The state of being sufficiently prepared for an intended submission.

IN USE assess submission readiness

dossier

An organized collection of documents supporting regulatory review.

IN USE maintain a coherent dossier

cross-reference

A pointer connecting a statement to supporting material.

IN USE verify the cross-reference

version control

Management of document revisions and their status.

IN USE maintain version control

regulatory commitment

An undertaking made in the relevant regulatory context.

IN USE track regulatory commitments

meeting minutes

The documented account of a meeting's discussion and outcomes.

IN USE reconcile meeting minutes

written response

A documented reply to a submitted question or issue.

IN USE review the written response

conditional advice

Advice whose applicability depends on stated circumstances.

IN USE preserve conditional advice

clinical hold

A regulatory order delaying or suspending a clinical investigation in the US context.

IN USE address a clinical hold

approval status

The actual state of a specified authorization decision.

IN USE verify approval status

indication

The disease, condition, or use for which a product is intended or authorized in context.

IN USE define the proposed indication

regulatory affairs

The function coordinating regulatory strategy, submissions, and interactions.

IN USE consult regulatory affairs

Precision check

Agency advice may address a specific question under stated assumptions. Preserve those limits in the meeting record. Do not report advice on a population or endpoint as approval of the complete program, final labeling, or a marketing application.

Phrases you can actually use

C Notice the language / 1 of 2

FOCUS THE QUESTION

"We seek advice on the proposed population and endpoint rationale."

SUPPLY THE REASON

"The rationale rests on the following evidence and assumptions."

AVOID BLANKET AGREEMENT

"This question does not ask for approval of the entire program."

DISTINGUISH STATUS

"The package is in preparation; no agency response has been received."

STATE THE SPONSOR VIEW

"Our proposed approach is set out in section three."

DISCLOSE INCOMPLETENESS

"Two supporting reports are not yet final."

CHECK THE REFERENCE

"Please verify that the cited section supports this statement."

PRESERVE A CONDITION

"Record the qualification alongside the advice, not in a separate summary."

Acceptable in what respect

Ask which specific proposal and evidence the question concerns.

Sponsor position

This is the company's proposal, not the authority's conclusion.

Final versus available

A draft can be available without being final or approved.

Phrases you can actually use

C Notice the language / 2 of 2

CONFIRM THE PATHWAY

"Regulatory affairs should confirm the applicable route for this product."

RESOLVE A DISCREPANCY

"The summary and supporting report currently describe different populations."

TRACK AN UNDERTAKING

"Identify the owner and due date for each recorded commitment."

MARK STUDY AUTHORITY

"Meeting preparation is not permission to begin the study."

SEPARATE SUBMISSIONS

"An IND and a marketing application serve different purposes."

CONTROL THE VERSION

"Use the agreed document version for the submitted package."

LIMIT THE CONCLUSION

"Advice on this question does not settle every development issue."

CLOSE THE RECORD

"Distinguish answered questions, conditions, and issues still open."

Advice versus authorization

Keep the applicable legal and procedural status explicit.

No objection

Do not paraphrase a limited response as unrestricted approval.

As proposed

This phrase ties the response to the details actually presented.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which revised question is most useful?

- A** Can we call unfinished reports final to simplify review?
- B** Does the agency agree with the proposed population and endpoint rationale described in section three?
- C** Is everything about the company acceptable forever?
- D** Will every future application be approved?

2. Which package statement is accurate?

- A** All supporting reports are final because the meeting is planned.
- B** No evidence needs to support a sponsor position.
- C** A scheduled meeting constitutes marketing authorization.
- D** Two reports remain unfinished and their status is identified.

3. Which distinction is correct in US terminology?

- A** An IND is a guarantee that an NDA will be approved.
- B** A meeting package replaces every required application.
- C** A BLA is merely a meeting calendar invitation.
- D** An IND supports clinical investigation; an NDA seeks marketing approval for a new drug.

4. How should limited advice be summarized?

- A** Expand it into approval of every proposed study.
- B** Delete qualifications to make the update easier to read.
- C** Treat the sponsor's preferred wording as the agency's conclusion.
- D** Preserve its subject, assumptions, conditions, and unresolved issues.

Answers and explanations: page 78.

A meeting question that asks for blanket agreement

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

conditional advice / focused question / cross-reference / investigational new drug application / briefing document / meeting minutes / information gap / regulatory pathway / sponsor position / approval status

- 01 **Evan:** Our lead question asks whether the whole development plan is acceptable. It is concise, but I suspect it will not give us the advice we need.
- 02 **Priya:** We need a ¹ _____. The immediate issue is the proposed patient population and endpoint rationale, not every aspect of the program from development through launch.
- 03 **Evan:** The team has a preferred approach. Should we present alternatives equally, or make clear which approach we are proposing and why we favor it?
- 04 **Priya:** State the ² _____ clearly, then give the rationale and relevant alternatives. Reviewers should not have to infer our proposal from a collection of background slides.
- 05 **Evan:** The supporting summaries are available, although two underlying reports are unfinished. I do not want the package to imply that those reports are final.
- 06 **Priya:** Identify that ³ _____ and explain its relevance. We should not conceal unfinished work or claim that a summary has the status of a completed report.
- 07 **Evan:** Section three explains the population. Another section uses a broader description, which could make our question seem different depending on where the reviewer starts.
- 08 **Priya:** Reconcile the wording in the ⁴ _____. A precise question is not enough if the supporting text describes a different proposal or leaves the study population ambiguous.
- 09 **Evan:** I will also check the references. One footnote points to an earlier table, and the current table no longer contains the information cited.
- 10 **Priya:** Verify each ⁵ _____ after the content is settled. A broken evidence trail forces reviewers to guess which material we intended and can weaken an otherwise useful discussion.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Evan:** Some colleagues are calling the meeting package our IND approval package. That seems to mix the meeting purpose with the status of a regulatory submission.
- 12 **Priya:** Use the terms carefully. An ⁶ _____ concerns proposed clinical investigation in the US context. Preparing this meeting package does not authorize the study or establish a marketing approval.
- 13 **Evan:** The long-range plan also mentions an NDA or BLA. We should explain the applicable route without suggesting that either application is already ready.
- 14 **Priya:** Exactly. The ⁷ _____ depends on the product and context. Regulatory affairs should confirm the route, while the current discussion remains focused on the specific development question.
- 15 **Evan:** After the meeting, the executive team will want a short update. How do we keep it concise without losing qualifications that affect the advice?
- 16 **Priya:** Preserve any ⁸ _____ in the main summary. If a response depends on the proposed population or additional evidence, that condition belongs beside the conclusion, not buried elsewhere.
- 17 **Evan:** We should compare our notes with the official record through the applicable process. A colleague's enthusiastic recollection should not become the only account.
- 18 **Priya:** Yes, reconcile the ⁹ _____ and track unresolved differences. Assign owners to follow-up items and commitments rather than treating a successful conversation as completion of all required work.
- 19 **Evan:** I will revise the package to separate the proposal, evidence, questions, and missing reports. Our internal update will say that no response has yet been received.
- 20 **Priya:** And keep ¹⁰ _____ explicit. Advice on one issue is not blanket program approval, and preparation for a meeting is not permission to start the study.

ANSWERS / EXPLANATIONS: PAGE 79

1. focused question | 2. sponsor position | 3. information gap | 4. briefing document | 5. cross-reference | 6. investigational new drug application | 7. regulatory pathway | 8. conditional advice | 9. meeting minutes | 10. approval status

The response answers one question

F Say it

An invented meeting note supports an endpoint approach only for the population described in the package. It requests further justification of follow-up duration. It grants no marketing authorization.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

conditional / population / unresolved / authorization

Regulatory lead: "The endpoint advice is ¹ _____."

Clinical lead: "Follow-up duration remains ² _____."

Regulatory lead: "Preserve the stated ³ _____."

Clinical lead: "This is not marketing ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 79

1. conditional | 2. unresolved | 3. population | 4. authorization

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Two visit windows and no confirmed controlling version

A Read the situation

Resolve conflicting trial instructions through an explicit, documented clarification rather than choosing a convenient version.

Coordinator Maya is scheduling next week. A protocol attachment gives a plus-or-minus-three-day visit window; a site quick-reference sheet gives seven. The controlling version and site implementation status are unconfirmed. No participant awaits an urgent care decision. Clinical research associate Ben must obtain clarification through the investigator and sponsor process. Neither may invent a window, silently change records, or assume the newest attachment governs the site.

PEOPLE / Maya: Site coordinator | Ben: Clinical research associate

READ BACK THE CONFLICT EXACTLY

A useful escalation states both wordings, identifies their sources, and asks a specific question about the applicable version. Avoid describing the issue merely as confusion at the site. Precise language helps colleagues resolve the document problem without assigning blame or improvising clinical instructions.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is the documented conflict?

- A** Two confirmed identical protocol versions
- B** A participant refusing all care
- C** A completed decision approving a new window
- D** Three-day versus seven-day visit windows in two sources

2. What has not been confirmed?

- A** Whether a quick-reference sheet exists
- B** Whether the two wordings differ
- C** The controlling version and its implementation status at the site
- D** Whether appointments are being prepared for next week

3. Which response avoids an unauthorized assumption?

- A** Rewrite historical visit records to match the preferred version.
- B** Escalate the exact discrepancy for documented clarification through the applicable process.
- C** Select seven days because it is more convenient.
- D** Treat the newest email attachment as automatically controlling.

Answers and explanations: page 80.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

protocol

The document specifying a clinical study's objectives, design, and procedures.

IN USE follow the applicable protocol

protocol amendment

A documented change to a study protocol.

IN USE implement an authorized protocol amendment

visit window

The allowed timing range for a specified study visit or assessment.

IN USE confirm the visit window

schedule of assessments

The planned timing of study procedures and measurements.

IN USE reconcile the schedule of assessments

controlling version

The authorized document version applicable to the work in question.

IN USE identify the controlling version

effective date

The date from which a document or change applies in context.

IN USE verify the effective date

site activation

Confirmation that a site meets the applicable conditions to begin specified study activity.

IN USE confirm site activation status

principal investigator

The investigator responsible for leading the study at a site.

IN USE consult the principal investigator

clinical research associate

A professional supporting study oversight and monitoring within assigned responsibilities.

IN USE contact the clinical research associate

study coordinator

A site professional coordinating assigned trial activities.

IN USE brief the study coordinator

good clinical practice

Principles and standards for ethical, reliable clinical trial conduct.

IN USE apply good clinical practice

informed consent

The voluntary agreement process based on adequate study information and understanding.

IN USE document the informed-consent process

Precision check

A later file date does not establish that a document is approved and effective for a particular site. Confirm identity, version, authorization, and applicability. A quick-reference sheet should not silently replace the controlling protocol or become an unofficial amendment.

The vocabulary that does the work

B Find the words / 2 of 2

03

Read the definitions and say the common word combinations aloud.

institutional review board

A body reviewing research involving human participants in the US context.

IN USE confirm institutional review board review

eligibility criterion

A condition determining whether someone may enter a study.

IN USE verify eligibility criteria

randomization

Allocation to study groups using a chance-based method.

IN USE protect the randomization process

blinding

Concealment of treatment allocation from specified parties.

IN USE maintain blinding

source record

The original record or certified copy containing relevant study observations.

IN USE preserve the source record

electronic case report form

An electronic tool for collecting protocol-required participant data.

IN USE complete the electronic case report form

data query

A documented request to clarify or resolve a data issue.

IN USE resolve a data query

protocol deviation

A departure from the study protocol.

IN USE assess a potential protocol deviation

monitoring finding

An issue identified during study monitoring.

IN USE document a monitoring finding

audit trail

A traceable record of changes and associated information.

IN USE preserve the audit trail

training record

Documentation of completed instruction or qualification.

IN USE verify the training record

essential record

A record supporting evaluation of trial conduct and result reliability.

IN USE maintain essential records

Precision check

GCP means good clinical practice; CRA, clinical research associate; PI, principal investigator; IRB, institutional review board; and eCRF, electronic case report form. Roles and approval processes vary. These exercises practice clarification, not instructions to alter care, consent, or study procedures.

Phrases you can actually use

C Notice the language / 1 of 2

03

STATE THE DISCREPANCY

"The protocol attachment says three days; the quick-reference sheet says seven."

IDENTIFY THE SOURCE

"I will include the document identifiers and exact sections."

ESCALATE PRECISELY

"Please resolve the timing discrepancy through the investigator and sponsor process."

SEPARATE STATUS

"Received by email does not necessarily mean approved for implementation."

REQUEST THE AUTHORITY

"Which approved version is effective for this site?"

AVOID A GUESS

"I cannot confirm the visit window from these conflicting files."

PROTECT THE RECORD

"We must preserve the original entry and any documented correction history."

CHECK TRAINING

"Has the relevant site team completed the required update?"

Applicable

A document can be approved but not yet applicable to a given site or activity.

Clarification versus amendment

Explaining an existing requirement differs from changing it.

Received versus implemented

Receipt of a file does not prove authorized implementation or training.

Phrases you can actually use

C Notice the language / 2 of 2

03

LIMIT THE RESPONSE

"I am confirming the governing instruction, not proposing a new window."

ASSESS IMPACT

"After confirmation, determine whether any completed visits require review."

KEEP CARE DISTINCT

"Administrative clarification must not replace appropriate clinical judgment."

CONTROL OBSOLETE COPIES

"Handle superseded quick-reference material through the document process."

CLOSE THE LOOP

"Please acknowledge the clarification and identify any affected appointments."

AVOID BLAME

"The two sources conflict; let us establish the applicable instruction."

DOCUMENT THE OUTCOME

"Record the confirmed version, effective status, and source of clarification."

CONFIRM UNDERSTANDING

"Please read back the confirmed window and the document it comes from."

Source versus eCRF

These records have related but distinct roles; discrepancies need traceable resolution.

Potential deviation

Do not classify the event definitively before the governing facts are established.

Read-back

Repeat the key instruction and source to expose misunderstanding before action.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which clarification request is strongest?

- A** Can the newest attachment replace approval evidence?
- B** Please confirm the approved site-effective version and section governing the visit window.
- C** Which window will make our calendar easiest?
- D** May we ignore both documents and choose five days?

2. What does a newer file date prove by itself?

- A** That all required training is complete
- B** That older source records may be overwritten
- C** Only that the file carries a later date, not that it governs the site
- D** That every site has implemented the document

3. How should a necessary record correction be handled?

- A** Through the applicable traceable correction process, preserving the original history
- B** By silently replacing the original value
- C** By deleting the discrepancy from the record
- D** By changing all records before checking the protocol

4. What should follow an authorized clarification?

- A** Declare every prior visit compliant without review.
- B** Treat clarification as permission to change any unrelated procedure.
- C** Confirm understanding and assess affected appointments or records through the appropriate process.
- D** Assume no further communication is needed.

Answers and explanations: [page 80](#).

Two visit windows and no confirmed controlling version

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

controlling version / audit trail / training record / schedule of assessments / protocol deviation / effective date / read-back / essential records / protocol amendment / principal investigator

- 01 **Maya:** I am scheduling next week's visits and have two different windows. The protocol attachment says plus or minus three days; the quick-reference sheet says seven.
- 02 **Ben:** Please send the identifiers and exact sections for both. We need the ¹ _____ before confirming the schedule. I cannot choose a window from the filenames alone.
- 03 **Maya:** The seven-day sheet arrived in a newer email. The coordinator who forwarded it assumed that made it the current instruction for every site.
- 04 **Ben:** A later email does not establish the ² _____ or implementation status. We should confirm which approved document applies here through the investigator and sponsor process.
- 05 **Maya:** I will quote both wordings in the request. Calling it a scheduling question without the numbers would make the reviewer reconstruct the discrepancy.
- 06 **Ben:** Include the relevant ³ _____ references as well. The team needs enough context to identify whether the quick-reference material reflects an authorized change or an error.
- 07 **Maya:** Nobody is waiting for an urgent care decision. This concerns next week's appointments, but I want to resolve it before we send instructions to participants.
- 08 **Ben:** Ask the ⁴ _____ to handle the site issue through the appropriate route. We should not invent clinical instructions or let an administrative document question substitute for medical judgment.
- 09 **Maya:** If the seven-day wording came from an amendment, we need more than a copy of the amended page. We need to know its status here.
- 10 **Ben:** Correct. A ⁵ _____ has an applicable approval and implementation process. Confirm those facts rather than treating circulation of the file as evidence that every required step occurred.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Maya:** The quick-reference sheet is useful at the desk, but it cannot create a separate set of rules. We should reconcile it after the answer is confirmed.
- 12 **Ben:** And verify the relevant ⁶ _____. Even a correct document may not resolve the operational problem if the people using it have not received the required update.
- 13 **Maya:** What about earlier visits? We should not declare them compliant or noncompliant until we know which instruction actually applied at the time.
- 14 **Ben:** Exactly. Assess any potential ⁷ _____ through the appropriate process after establishing the facts. Do not assume that today's clarification automatically governs every historical visit.
- 15 **Maya:** One colleague suggested changing the old dates in the tracker to remove the apparent conflict. That would conceal what happened rather than resolve it.
- 16 **Ben:** Preserve the ⁸ _____. Any necessary correction must follow the applicable process and retain traceability. We must not rewrite the history to make the schedule appear consistent.
- 17 **Maya:** Once we receive the response, I will send a short site update with the confirmed version and the appointments that need attention.
- 18 **Ben:** Use a ⁹ _____ for the critical timing instruction. Ask the relevant colleague to repeat the window and its source so we can detect a misunderstanding before acting on it.
- 19 **Maya:** I will keep the clarification with the study records and route any affected data questions separately. We should not leave the answer only in someone's inbox.
- 20 **Ben:** Good. The ¹⁰ _____ should show the discrepancy, the authorized resolution, and the follow-up. That closes the communication loop without turning a convenient reference sheet into an unofficial protocol.

ANSWERS / EXPLANATIONS: PAGE 81

1. controlling version | 2. effective date | 3. schedule of assessments | 4. principal investigator | 5. protocol amendment | 6. training record | 7. protocol deviation | 8. audit trail | 9. read-back | 10. essential records

Receipt does not establish applicability

F Say it

A coordinator receives version 5 by email. The site record still identifies version 4 as effective. No implementation confirmation for version 5 has been supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unconfirmed / received / recency / confirmation

Coordinator: "Version 5 has been ¹ _____."

Monitor: "Its site applicability remains ² _____."

Coordinator: "I will request implementation ³ _____."

Monitor: "Do not infer authority from file ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 81

1. received | 2. unconfirmed | 3. confirmation | 4. recency

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A favorable secondary result takes over the headline

04

A Read the situation

Explain a trial readout without replacing the prespecified question or overstating statistical certainty.

A fictional trial measures week-12 symptoms; lower scores are better. The primary treatment-minus-control estimate is -2 points (95% confidence interval: -5 to +1; $p=0.19$). The prespecified two-sided threshold is 0.05. A secondary endpoint has nominal $p=0.01$, but the testing plan requires primary success before a confirmatory secondary claim. Statistician Anika and medical lead Luis must correct the headline: the trial proved efficacy.

PEOPLE / Anika: Statistician | Luis: Medical lead

DO NOT REPAIR A RESULT BY CHANGING THE QUESTION

Lead with the prespecified primary result and explain uncertainty before highlighting other findings. A disappointing primary test does not make every observation worthless, but a favorable secondary result does not erase the testing plan. Keep statistical evidence and clinical importance separate in the discussion.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Did the primary analysis meet the specified significance threshold?

A No; $p=0.19$ is above 0.05.

B Yes; every negative point estimate is significant.

C Yes; the secondary p -value replaces it.

D No; because the confidence interval contains only negative values.

2. What is the secondary endpoint's status under this plan?

A A guarantee of clinical benefit in every patient

B A nominal result that cannot support the stated confirmatory claim after primary failure

C Automatic proof that the primary endpoint succeeded

D A replacement primary endpoint chosen after the readout

3. What does the negative point estimate indicate?

A The treatment is worse because negative numbers are always unfavorable.

B The confidence interval excludes no difference.

C The estimated difference favors treatment on this lower-is-better scale.

D The trial has proved no possible benefit.

Answers and explanations: page 82.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

primary endpoint

The outcome designated to address the study's principal objective.

IN USE report the primary endpoint

secondary endpoint

An additional prespecified outcome supporting other study objectives.

IN USE interpret secondary endpoints

exploratory endpoint

An outcome analyzed to investigate questions without the same confirmatory status.

IN USE label exploratory endpoints

estimand

The precise treatment-effect target defined for a clinical question.

IN USE specify the estimand

intercurrent event

An event after treatment starts that affects interpretation or existence of relevant measurements.

IN USE address intercurrent events

treatment-policy strategy

An estimand approach targeting outcomes regardless of specified intercurrent events.

IN USE define a treatment-policy strategy

hypothetical strategy

An estimand approach targeting a scenario in which specified intercurrent events would not occur.

IN USE justify a hypothetical strategy

analysis population

The defined group included in an analysis.

IN USE specify the analysis population

statistical analysis plan

The detailed prespecified methods for study analyses.

IN USE follow the statistical analysis plan

point estimate

A single numerical estimate of a population quantity.

IN USE present the point estimate

confidence interval

An interval from a procedure designed to cover the target parameter at a stated long-run rate.

IN USE report the confidence interval

p-value

The probability, under the null model, of a result at least as incompatible with it as observed.

IN USE interpret the p-value

Precision check

The estimate favors treatment, but the interval includes zero and the primary p-value exceeds 0.05. This is not proof of no effect. It is also not a successful primary test. The secondary finding retains its actual status under the prespecified plan.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

significance threshold

The prespecified criterion used for a statistical test decision.

IN USE apply the significance threshold

multiplicity

The issue of repeated testing increasing opportunities for false positive conclusions.

IN USE control multiplicity

testing hierarchy

A prespecified order and conditions for confirmatory tests.

IN USE respect the testing hierarchy

nominal p-value

A p-value reported without the relevant multiple-testing adjustment or confirmatory protection.

IN USE label a nominal p-value

effect size

The magnitude of a difference or relationship on a stated scale.

IN USE interpret the effect size

clinical importance

The relevance of an effect to patients or clinical decisions.

IN USE assess clinical importance

missing data

Required or relevant observations not available for analysis.

IN USE assess missing data

sensitivity analysis

An analysis examining robustness to assumptions for the same target question.

IN USE perform sensitivity analysis

subgroup analysis

Examination of results within defined subsets of participants.

IN USE qualify a subgroup analysis

post hoc analysis

An analysis developed after the relevant data were examined.

IN USE label a post hoc analysis

data cutoff

The date or point defining the data included in a readout.

IN USE state the data cutoff

topline readout

An initial summary of key study results.

IN USE prepare a balanced topline readout

Precision check

An estimand defines treatments, population, outcome, intercurrent-event handling, and summary measure. An analysis method estimates that target. Changing how discontinuation is handled can change the question, not merely the numerical answer.

Phrases you can actually use

C Notice the language / 1 of 2

LEAD WITH THE PRIMARY

"The primary analysis did not meet the prespecified significance threshold."

PRESERVE UNCERTAINTY

"The confidence interval includes no difference."

RESPECT THE PLAN

"Primary success was required before a confirmatory secondary claim."

SEPARATE IMPORTANCE

"Statistical significance and clinical importance are different questions."

STATE THE DIRECTION

"The point estimate favors treatment on this lower-is-better scale."

QUALIFY THE SECONDARY

"The secondary p-value is nominal under this testing plan."

AVOID A FALSE NEGATIVE CLAIM

"Failure to meet the threshold does not prove there is no effect."

DEFINE THE TARGET

"Which treatment effect does the estimand specify?"

Nominal

Do not omit this qualifier when multiplicity protection does not support the claim.

Not significant

This does not mean the effect is known to be exactly zero.

Minus two points

Name the comparison direction and whether lower values are favorable.

Phrases you can actually use

C Notice the language / 2 of 2

CHECK THE POPULATION

"Confirm which participants are included in this analysis."

ASK ABOUT MISSINGNESS

"What assumptions support the handling of missing outcomes?"

CLARIFY THE SCALE

"The difference is treatment minus control, measured in score points."

AVOID A RESCUE HEADLINE

"A favorable secondary result does not replace the primary outcome."

HANDLE EVENTS EXPLICITLY

"State how treatment discontinuation affects the target question."

LABEL LATER WORK

"This post hoc analysis was not the prespecified primary analysis."

KEEP DATES VISIBLE

"Identify the data cutoff for this readout."

CLOSE WITH THE FULL RESULT

"Present the estimate, uncertainty, testing status, and clinical context together."

Confidence interval

Do not describe this frequentist interval as a 95% probability that this fixed parameter lies inside it.

Estimand versus estimator

The estimand is the target; the estimator is the method used to estimate it.

Post hoc

Later exploration can be useful, but it must retain its actual evidential status.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which headline is most accurate?

- A** Trial proved efficacy because one p-value was 0.01.
- B** Treatment has exactly zero effect in every patient.
- C** The secondary endpoint is now the primary endpoint.
- D** Primary threshold not met; secondary finding is nominal under the prespecified plan.

2. What does the interval from -5 to +1 show?

- A** It includes values favoring treatment, zero, and values favoring control on this scale.
- B** It excludes every possibility of no difference.
- C** It contains only unfavorable treatment differences.
- D** It proves that 95% of patients improved by two points.

3. Why does clinical importance need separate discussion?

- A** A negative sign always proves a meaningful improvement.
- B** Clinical importance is identical to the number of endpoints.
- C** The practical meaning of a score difference is not established by a p-value alone.
- D** Any p-value below one guarantees patient benefit.

4. Which statement about later subgroup work is appropriate?

- A** Assume smaller groups cannot produce chance findings.
- B** Label it according to its actual prespecification and multiplicity status.
- C** Present it as the original primary analysis regardless of timing.
- D** Hide the overall trial result whenever a subgroup looks favorable.

Answers and explanations: [page 82](#).

A favorable secondary result takes over the headline

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

topline readout / primary endpoint / point estimate / confidence interval / estimand / intercurrent event / nominal p-value / sensitivity analyses / testing hierarchy / Clinical importance

- 01 **Luis:** The draft headline says the trial proved efficacy. The supporting slide leads with the secondary endpoint's p-value of point zero one and moves the primary result below it.
- 02 **Anika:** We need to lead with the ¹ _____. Its p-value is point one nine, above the prespecified threshold of point zero five. The primary test did not succeed.
- 03 **Luis:** The estimated difference is minus two points. Since lower symptom scores are better, that direction favors treatment, even though the test did not meet the threshold.
- 04 **Anika:** Correct. Keep the ² _____ and its uncertainty together. We should not erase the observed direction, but neither should we turn a favorable estimate into a definitive efficacy conclusion.
- 05 **Luis:** The interval runs from minus five to plus one. That includes no difference and some values in the opposite direction, not just improvements.
- 06 **Anika:** State the ³ _____ plainly. It is not evidence that every patient improved by two points, and it does not establish that the true effect must be exactly zero.
- 07 **Luis:** Can the secondary result support the headline independently? The plan says it can be confirmatory only after primary success, which did not occur.
- 08 **Anika:** Then the ⁴ _____ prevents that interpretation. We cannot change the sequence after seeing the readout simply because another result is more attractive.
- 09 **Luis:** We can still report the secondary observation, provided we describe its status. Hiding it would not make the primary explanation more complete.
- 10 **Anika:** Yes, report its ⁵ _____ with the relevant qualification. The problem is not mentioning the result; it is implying a level of confirmatory support the plan does not provide.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Luis:** The medical team will also ask whether a two-point change matters clinically. The statistical slide alone does not answer that question.
- 12 **Anika:** Exactly. ⁶ _____ depends on the scale, context, and supporting evidence. A significance label is not a substitute for explaining what the measured difference could mean to patients.
- 13 **Luis:** Before finalizing the interpretation, I want to confirm the treatment-effect question. Colleagues have used different language about participants who stopped the assigned treatment.
- 14 **Anika:** Return to the ⁷ _____. We need the defined population, outcome, treatments, event-handling strategy, and summary measure, not an informal description that changes from one slide to another.
- 15 **Luis:** So discontinuation is not merely a nuisance to remove from the dataset. Its treatment may change which question the analysis answers.
- 16 **Anika:** It can be an ⁸ _____. Distinguish the target question from missing observations and explain the planned approach; do not assume every discontinuation should be handled in the same way.
- 17 **Luis:** Additional analyses may help us understand how dependent the result is on assumptions. We should avoid choosing only the most favorable alternative.
- 18 **Anika:** Use the planned ⁹ _____ and label other work accurately. A later subgroup finding does not become the primary result, and a different question should not be presented as the same analysis.
- 19 **Luis:** I will replace the headline and show the primary estimate, interval, and test status first. The secondary finding will retain its qualification.
- 20 **Anika:** That will make the ¹⁰ _____ balanced and intelligible. We can discuss further investigation without either claiming proven efficacy or pretending that an unsuccessful significance test proves the absence of every possible effect.

ANSWERS / EXPLANATIONS: PAGE 83

1. primary endpoint | 2. point estimate | 3. confidence interval | 4. testing hierarchy | 5. nominal p-value | 6. Clinical importance | 7. estimand | 8. intercurrent event | 9. sensitivity analyses | 10. topline readout

Relative and absolute reductions

F Say it

In a separate fictional comparison, events occur in 200 of 1,000 control participants and 150 of 1,000 treatment participants. Use only these counts; no significance result is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

20% / 15% / 25% / five percentage points

Analyst: "The control event rate is ¹ _____."

Reviewer: "The treatment event rate is ² _____."

Analyst: "The absolute reduction is ³ _____."

Reviewer: "Relative to control, the reduction is ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 83

1. 20% | 2. 15% | 3. five percentage points | 4. 25%

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

An incomplete safety report

A Read the situation

Preserve reported facts, avoid premature causality judgments, and route a safety report without waiting for perfect information.

05

Medical-information associate Nora receives a message from a clinician reporting dizziness after a patient used fictional product LM-4. The clinician calls the dizziness severe, but the message does not state whether hospitalization, a life-threatening condition, or another seriousness criterion applies. Dates, dose, outcome, and other medicines are missing. The company procedure requires prompt internal routing of available safety information and documented follow-up. Safety colleague Rafael must distinguish reported severity, unconfirmed seriousness, and unassessed causality. This conversation is an internal handoff, not advice to the patient.

PEOPLE / Nora: Medical-information associate | Rafael: Safety colleague

KEEP THE REPORT AND THE JUDGMENT SEPARATE

Use reported for the clinician's words and unconfirmed for missing facts. Do not delay the required internal handoff while trying to prove that a product caused the event. Equally, do not dismiss the report because causality is uncertain. Qualified safety assessment and clinical care have their own appropriate routes.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What has the clinician actually reported?

- A Dizziness described as severe after product use
- C A completed causality assessment

- B Confirmed product-caused hospitalization
- D A verified outcome and dosing history

2. Can severe automatically be treated as serious in the regulatory sense?

- A Yes; the words are always interchangeable.
- C No; severe events must never be routed.

- B Yes; any adjective establishes hospitalization.
- D No; intensity and seriousness criteria are different concepts.

3. What does the stated company procedure require?

- A Waiting until causality is proved before any handoff
- C Giving the patient a dosing recommendation during the handoff

- B Deleting reports with missing dates
- D Prompt internal routing of available information with documented follow-up

Answers and explanations: [page 84](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

pharmacovigilance

Activities concerned with detecting, assessing, understanding, and preventing medicine-related problems.

IN USE support pharmacovigilance

adverse event

An unfavorable medical occurrence reported during product use, without necessarily established causation.

IN USE capture an adverse event

adverse reaction

An undesirable response for which a causal relationship is at least reasonably suspected in context.

IN USE assess a suspected adverse reaction

seriousness

Classification based on applicable outcomes or medical criteria, not intensity alone.

IN USE assess seriousness

severity

The intensity of an event or symptom.

IN USE record reported severity

causality assessment

Evaluation of whether and how a product may be related to an event.

IN USE document causality assessment

Precision check

Severe describes intensity. Serious uses applicable criteria, including death, life-threatening events, hospitalization, disability, congenital anomalies, and other medically important conditions. Clarify missing facts; do not infer seriousness from intensity alone.

expectedness

Whether the nature or severity of an event is consistent with the applicable reference safety information.

IN USE evaluate expectedness

reference safety information

The designated information used for defined safety assessments in context.

IN USE identify reference safety information

safety signal

Information suggesting a possible new or changed risk that warrants further evaluation.

IN USE evaluate a safety signal

case intake

Initial receipt and recording of a safety report.

IN USE complete case intake

individual case safety report

A structured account of an individual suspected safety case.

IN USE prepare an individual case safety report

reporter

The person or source providing event information.

IN USE preserve reporter details

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

verbatim term

The exact wording used by the original reporter.

IN USE retain the verbatim term

event onset

The time an event began.

IN USE confirm event onset

temporal association

A relationship in timing without necessarily proving causation.

IN USE distinguish temporal association from causation

concomitant medication

Another medicine used during the relevant period.

IN USE document concomitant medications

dechallenge

What happens after a suspected product is withdrawn or reduced, when this occurs clinically.

IN USE record available dechallenge information

rechallenge

What happens after re-exposure to a product, when this has occurred clinically.

IN USE record reported rechallenge information

follow-up request

A request for additional information about an existing case.

IN USE issue a focused follow-up request

case reconciliation

Checking that related records are consistent and accounted for across sources.

IN USE perform case reconciliation

duplicate report

Another report concerning a case already recorded.

IN USE investigate a possible duplicate report

MedDRA

Medical Dictionary for Regulatory Activities, a standardized terminology used for regulatory medical information.

IN USE apply appropriate MedDRA coding

benefit-risk assessment

Evaluation of favorable and unfavorable effects in the relevant context.

IN USE update the benefit-risk assessment

aggregate safety review

Evaluation of safety information across cases or datasets.

IN USE conduct an aggregate safety review

Precision check

Dechallenge and rechallenge describe actual clinical events, not instructions to change treatment. Preserve reported facts, follow the safety process, and route clinical questions to qualified treating professionals.

Phrases you can actually use

C Notice the language / 1 of 2

OPEN THE HANDOFF

"I have an incomplete report that needs safety intake."

SEPARATE THE CLASSIFICATION

"Seriousness has not yet been confirmed."

NAME THE GAPS

"Dates, dose, outcome, and concomitant medicines are missing."

KEEP FOLLOW-UP FOCUSED

"Please clarify whether any applicable seriousness criterion was met."

PRESERVE THE WORDING

"The clinician described the dizziness as severe."

AVOID CAUSALITY OVERREACH

"The event followed use; causality has not been assessed."

ROUTE PROMPTLY

"I will send the available information through the required internal channel now."

PROTECT THE ORIGINAL

"Retain the reporter's exact words alongside any coded term."

Reported versus confirmed

Attribute unverified facts to their source.

Unknown versus no

Missing hospitalization information is not evidence that hospitalization did not occur.

After versus because of

Timing alone does not establish product causation.

Phrases you can actually use

C Notice the language / 2 of 2

CLARIFY TIMING

"When did the event begin relative to product use?"

ASSIGN OWNERSHIP

"Who will make the follow-up request and track the response?"

AVOID DISMISSAL

"Missing information does not justify discarding the report."

SEPARATE CARE

"Clinical treatment questions belong with the qualified treating professional."

MARK AN UNKNOWN

"The outcome is unknown, not recovered."

CONFIRM RECEIPT

"Please acknowledge intake so we know the handoff reached the safety team."

DISTINGUISH A SIGNAL

"One report does not by itself establish a confirmed causal risk."

CLOSE ACCURATELY

"Record what was reported, what remains unknown, and the next responsible owner."

Serious versus severe

Preserve the original wording while clarifying the applicable classification.

Duplicate

Check the connection; do not discard similar reports merely because their wording resembles an earlier case.

Prompt internal routing

Follow the actual procedure and applicable timelines, not a deadline invented for this language exercise.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which handoff is most precise?

- A** No serious event occurred because hospitalization was not mentioned.
- B** The patient recovered because no outcome was supplied.
- C** Severe dizziness was reported after LM-4 use; seriousness and causality remain unassessed.
- D** LM-4 definitely caused a serious reaction.

2. What should happen while missing details are sought?

- A** Delete the original wording after choosing a code.
- B** Give a medication-change instruction to improve the report.
- C** Route available information promptly under the stated procedure and document follow-up.
- D** Keep the report outside the safety process until every field is complete.

3. Which follow-up asks for a missing fact without leading the reporter?

- A** Please confirm the product was definitely responsible.
- B** Can you change severe to mild so the case is simpler?
- C** Since the patient recovered, can we close the case?
- D** Was the patient hospitalized, and what was the outcome?

4. What does a temporal association establish?

- A** That the event and product use have a stated timing relationship, not necessarily causation
- B** That the product caused the event in every similar patient
- C** That a label update is automatically required
- D** That further assessment is unnecessary

Answers and explanations: [page 84](#).

An incomplete safety report

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

05

WORD + PHRASE BANK

temporal association / concomitant medications / verbatim term / case intake / severity / follow-up request / receipt confirmation / case reconciliation / benefit-risk assessment / safety signal

- 01 **Nora:** I received a clinician's message about dizziness after LM-4 use. The clinician called it severe, but several details are missing. I want to route it correctly.
- 02 **Rafael:** Send the available information through ¹ _____ now, as our procedure requires. We can document what is unknown and arrange follow-up without waiting for you to complete a safety assessment yourself.
- 03 **Nora:** I have kept the clinician's wording. There is no statement about hospitalization, a life-threatening condition, or another criterion that would establish seriousness.
- 04 **Rafael:** Keep ² _____ separate from seriousness. Severe describes intensity; it does not settle the regulatory classification. We need to clarify the relevant facts rather than translate one word directly into the other.
- 05 **Nora:** The event occurred after product use, but the message gives no exact dates. I cannot tell how close the timing was or whether other explanations were considered.
- 06 **Rafael:** Describe a reported ³ _____, not proven causation. The safety assessment will consider the available evidence. Do not add a causal conclusion that the source did not establish.
- 07 **Nora:** We are missing dose, outcome, and other medicines. I will list those gaps so they remain visible in the structured record.
- 08 **Rafael:** Include ⁴ _____ in the follow-up request. Record unknown fields as unknown; absence of information must not become an assertion that no other medicine was used.
- 09 **Nora:** The clinician's exact phrase is severe dizziness. Should I replace it with a preferred term before sending the message, so the record looks consistent?
- 10 **Rafael:** Preserve the ⁵ _____. Appropriate coding can be added through the process, but the original wording remains important. Standardization should not erase what the reporter actually said.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

- 11 **Nora:** I can ask when the dizziness started, whether it resolved, and whether hospitalization occurred. I should avoid asking the clinician to confirm a conclusion we prefer.
- 12 **Rafael:** Make the ⁶ _____ neutral and specific. Ask for the facts needed to assess the case, and document the request and response through the required channel.
- 13 **Nora:** Who owns that contact? I do not want both teams to approach the reporter independently and create two incomplete versions of the same conversation.
- 14 **Rafael:** We will assign an owner and use ⁷ _____ to keep the records aligned. If another report arrives, assess whether it is connected rather than assuming similar wording means a duplicate.
- 15 **Nora:** The commercial team may ask whether this is a new safety signal. I do not think this handoff alone answers that broader question.
- 16 **Rafael:** Correct. A ⁸ _____ requires evaluation in context. This individual report contributes information, but we should not announce a confirmed new risk or dismiss it without assessment.
- 17 **Nora:** I also want the internal update to avoid implying that a label change has been decided. We are much earlier in the information process.
- 18 **Rafael:** Any ⁹ _____ or labeling action belongs in the qualified review process. Our immediate task is accurate capture, appropriate routing, and follow-up, not deciding the product's overall profile.
- 19 **Nora:** I will route the original message, identify every missing field, and note the agreed follow-up owner. Clinical treatment questions will stay with the qualified treating professional.
- 20 **Rafael:** Please request ¹⁰ _____ as well. That gives us a closed handoff rather than an assumption that sending a message means the information reached the right team.

ANSWERS / EXPLANATIONS: PAGE 85

1. case intake | 2. severity | 3. temporal association | 4. concomitant medications | 5. verbatim term | 6. follow-up request | 7. case reconciliation | 8. safety signal | 9. benefit-risk assessment | 10. receipt confirmation

Unknown is not negative

F Say it

A report says a rash occurred after product use. It does not state hospitalization, outcome, or a causal assessment. The applicable internal process requires routing available information now.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

now / follow-up / unknown / causation

Associate: "Hospitalization status is ¹ _____."

Reviewer: "The timing does not establish ² _____."

Associate: "I will route the available report ³ _____."

Reviewer: "Record the missing details for ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 85

1. unknown | 2. causation | 3. now | 4. follow-up

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A gap in the batch record

A Read the situation

Report a quality-documentation gap precisely and separate investigation status from batch disposition.

Batch Q-214 lacks a verification entry; whether the check occurred is unknown. The batch remains under quality review, not released. Production lead Diego asks quality specialist Helen whether available laboratory results justify saying fully reviewed. Site procedure requires documented investigation and authorized disposition. Neither may backdate an entry, assume the check occurred, or promise release to meet a shipment date.

PEOPLE / Diego: Production lead | Helen: Quality specialist

06

DESCRIBE THE GAP WITHOUT DECIDING THE CAUSE

Missing documentation is a fact; assuming the activity happened or did not happen adds a conclusion. State the observed gap, current batch status, investigation owner, and next update. This is more useful to supply colleagues than either premature reassurance or an unsupported declaration that the product failed.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is established about batch Q-214?

- A** The product definitely failed every laboratory test.
- B** A verification entry is missing and quality review remains open.
- C** The batch has been released.
- D** The verification definitely occurred.

2. Why is fully reviewed inaccurate?

- A** A shipment date automatically closes review.
- B** A production request supplies quality authorization.
- C** The required investigation and authorized disposition are not complete.
- D** Available laboratory results make all records irrelevant.

3. Which action is excluded by the case?

- A** Backdating an entry to make the record appear complete
- B** Preserving the record
- C** Investigating the gap
- D** Giving supply an accurate status update

Answers and explanations: page 86.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

chemistry, manufacturing, and controls

Product and process information addressing composition, manufacture, and quality control.

IN USE assess CMC readiness

current good manufacturing practice

Applicable requirements and practices supporting consistent pharmaceutical quality.

IN USE maintain CGMP compliance

batch record

Documentation of the production and control activities for a specific batch.

IN USE review the batch record

master production instruction

The authorized instruction defining how a product is to be manufactured.

IN USE follow master production instructions

verification entry

A record showing a required check and its associated information.

IN USE complete the verification entry

documentation gap

Missing or incomplete recorded information.

IN USE investigate a documentation gap

data integrity

The completeness, consistency, and accuracy of data through its lifecycle.

IN USE protect data integrity

contemporaneous record

A record made when the relevant activity occurs.

IN USE maintain contemporaneous records

traceability

The ability to follow an activity, material, or result through its records.

IN USE preserve traceability

deviation investigation

A structured examination of a departure from an applicable requirement or procedure.

IN USE conduct a deviation investigation

root cause

The underlying cause supported by investigation.

IN USE establish the root cause

corrective action

Action addressing a detected problem and its cause in the relevant system.

IN USE implement corrective action

Precision check

Available test results are not the same as a completed batch review or release. The missing entry needs investigation under the applicable process. A signature added later must not falsely represent when an activity occurred; any permitted correction needs an accurate, traceable basis.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

preventive action

Action intended to prevent a potential problem from occurring.

IN USE define preventive action

CAPA

Corrective and preventive action, the system for addressing actual and potential quality problems.

IN USE verify CAPA effectiveness

effectiveness check

A review of whether an action achieved its intended result.

IN USE plan an effectiveness check

out-of-specification result

A test result outside established acceptance specifications.

IN USE investigate an out-of-specification result

out-of-trend result

A result inconsistent with an expected pattern, even when it may meet specifications.

IN USE evaluate an out-of-trend result

specification

Defined tests, procedures, and acceptance criteria for a material or product.

IN USE confirm the applicable specification

certificate of analysis

A document reporting specified test results for a material or batch.

IN USE verify the certificate of analysis

batch disposition

The authorized decision on how a batch may be used or handled.

IN USE document batch disposition

release authorization

Formal permission for a batch to proceed to the defined use or distribution.

IN USE verify release authorization

change control

The managed evaluation and authorization of proposed changes.

IN USE route a change through change control

stability program

Planned assessment of product quality over time under specified conditions.

IN USE maintain the stability program

supply constraint

A limit affecting availability or delivery.

IN USE communicate the supply constraint

Precision check

CMC means chemistry, manufacturing, and controls; CGMP means current good manufacturing practice; CAPA means corrective and preventive action. OOS means out of specification and OOT means out of trend. A documentation gap is not automatically an OOS test result or a proven product defect.

Phrases you can actually use

C Notice the language / 1 of 2

STATE THE OBSERVED GAP

"The verification entry is missing from the batch record."

REPORT THE STATUS

"The batch remains under quality review and is not released."

PROTECT INTEGRITY

"Do not backdate or reconstruct an entry as though it were contemporaneous."

NAME THE OWNER

"Quality will coordinate the documented investigation under the site process."

AVOID AN ASSUMED CAUSE

"We have not established whether the check occurred."

SEPARATE EVIDENCE

"Available laboratory results do not complete the documentation review."

REQUEST THE INVESTIGATION

"Please identify the evidence needed to resolve this discrepancy."

LIMIT THE SUPPLY UPDATE

"Release timing is unconfirmed pending authorized disposition."

06

Fully reviewed

Use only when the applicable review is actually complete.

Performed versus documented

The absence of an entry leaves a question; it does not settle what happened.

Backdate

Do not make a later entry appear to have been made earlier.

Phrases you can actually use

C Notice the language / 2 of 2

PRESERVE TRACEABILITY

"Any permitted correction must retain its source and history."

CHECK EFFECTIVENESS

"An action is not complete merely because a training session was scheduled."

AVOID BLANKET BLAME

"Establish the cause before assigning an unsupported explanation."

CONFIRM AUTHORIZATION

"Who has authority to make the batch-disposition decision?"

DISTINGUISH RESULTS

"This documentation issue is not itself an out-of-specification test result."

ESCALATE PRESSURE

"The shipment deadline does not change the evidence or release authority."

KEEP ACTIONS SEPARATE

"Correction of the record and prevention of recurrence are distinct tasks."

CLOSE WITH AN UPDATE

"We will provide the next status update without promising a release date."

06

OOS versus OOT

A specification failure and an unusual trend are different findings.

Investigation versus disposition

Finding the cause and authorizing batch use are related but separate steps.

Estimated versus authorized

A hoped-for shipment date is not release authorization.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which supply update is supported?

- A** Q-214 definitely failed manufacturing because a field is blank.
- B** Q-214 remains under quality review; release timing is unconfirmed.
- C** Q-214 is fully reviewed because laboratory results are available.
- D** Q-214 is released because a customer needs it tomorrow.

2. What does the missing entry prove by itself?

- A** That the check definitely did not occur
- B** That every test result is invalid
- C** That required recorded information is absent
- D** That the check definitely occurred

3. Which action preserves data integrity?

- A** Add yesterday's date to make today's entry look timely.
- B** Copy another batch's verification to fill the space.
- C** Remove the blank page so the review looks complete.
- D** Use the authorized correction process with an accurate, traceable account.

4. What should a preventive response address?

- A** A promise that no future quality issue is possible
- B** The supported cause and a way to check whether recurrence risk is reduced
- C** A convenient explanation chosen before investigation
- D** Only the visual appearance of the record

Answers and explanations: [page 86](#).

A gap in the batch record

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

root cause / release authorization / effectiveness check / contemporaneous record / deviation investigation / batch record / documentation gap / batch disposition / out-of-specification result / traceability

06

- 01 **Diego:** Supply wants a Q-214 update. Laboratory results are available, but the verification entry is missing. Can I say fully reviewed?
- 02 **Helen:** No. The ¹ _____ remains incomplete for review purposes, and the investigation is open. Available laboratory results do not resolve the missing verification or authorize release.
- 03 **Diego:** We do not know whether the check was undocumented or missed. I will not choose an explanation before we have evidence.
- 04 **Helen:** Call it a ² _____. That states the observed fact without deciding the cause. The investigation should establish what evidence exists and what conclusions it can actually support.
- 05 **Diego:** A colleague suggested adding the signature with yesterday's date. They see it as tidying the record because production is under pressure.
- 06 **Helen:** That would misrepresent a ³ _____. We must not backdate or fabricate evidence. Any permitted later correction needs the appropriate process and an accurate account of its timing and basis.
- 07 **Diego:** I will preserve the original and supporting records. We need to understand the sequence without losing evidence of the discrepancy.
- 08 **Helen:** Exactly. Protect ⁴ _____ throughout the investigation. A cleaner-looking page is not an improvement if it hides what was recorded, what changed, or why the change was made.
- 09 **Diego:** Is this an out-of-specification result? Supply uses that phrase broadly, but we have a missing entry rather than a test value.
- 10 **Helen:** An ⁵ _____ is a different finding. Use the precise term so colleagues understand the problem we actually have instead of assuming a laboratory failure that has not been established.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

- 11 **Diego:** We need an owner for the investigation and a realistic next update. I can make records and production personnel available through the site process.
- 12 **Helen:** Quality will coordinate the ⁶ _____. We should identify the evidence needed and document the findings before choosing a cause or claiming that the issue is resolved.
- 13 **Diego:** If the investigation finds a process weakness, simply asking everyone to be more careful may not address it. We need an action connected to the actual cause.
- 14 **Helen:** Yes. Establish the ⁷ _____ before prescribing the response. Training may be relevant in some circumstances, but we should not use it as a default explanation for every documentation problem.
- 15 **Diego:** The action plan should also show how we will know it worked. Finishing a task is not necessarily the same as preventing the same gap again.
- 16 **Helen:** Include an ⁸ _____ suited to the issue. The review should examine the intended result, not just confirm that a meeting occurred or a new form was issued.
- 17 **Diego:** Supply asks whether release tomorrow is likely. I can report the status, but cannot promise a decision quality has not made.
- 18 **Helen:** The ⁹ _____ remains pending under the authorized process. A shipment deadline does not change the evidence standard or transfer release authority to the production update.
- 19 **Diego:** Then I will say the batch is under review, the investigation is open, and the release date is unconfirmed. I will provide the next agreed status update.
- 20 **Helen:** That is accurate. Keep ¹⁰ _____ separate from expectations about timing. We can communicate the supply constraint clearly without calling an unfinished review complete or declaring a defect that the facts have not established.

06

ANSWERS / EXPLANATIONS: PAGE 87

1. batch record | 2. documentation gap | 3. contemporaneous record | 4. traceability | 5. out-of-specification result | 6. deviation investigation | 7. root cause | 8. effectiveness check | 9. batch disposition | 10. release authorization

A test report is not a release decision

F Say it

A certificate of analysis is available, but the batch-disposition record is pending. The current status says not released. A customer has requested shipment tomorrow.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

pending / released / authorization / results

Planner: "The certificate reports test ¹ _____."

Quality: "Batch disposition is still ² _____."

Planner: "The current status is not ³ _____."

Quality: "The customer request does not supply release ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 87

1. results | 2. pending | 3. released | 4. authorization

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Stronger wording changes the claim

A Read the situation

Identify an unsupported promotional implication and return a precisely scoped claim through the required review route.

A fictional approved source supplied for an internal exercise describes a lower mean symptom score versus placebo at week 12 in a defined adult study population. A promotional draft instead says immediate relief for every patient and places risk information in an unreadable footnote. No evidence supports immediate onset or benefit in every patient. The complete revised item requires medical, legal, and regulatory review before external use. Medical reviewer Sofia and brand colleague Adam must restore the supported scope without treating familiar wording or an approaching deadline as final approval.

PEOPLE / Sofia: Medical reviewer | Adam: Brand colleague

07

EXPLAIN WHAT CHANGED IN THE MEANING

A review comment is more useful when it identifies the unsupported implication and the evidence boundary, rather than merely saying noncompliant. Offer wording tied to the supplied source, keep risk information appropriately visible, and identify the decision still required before the material can be used.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which part of the draft is unsupported by the supplied source?

- | | |
|---|---|
| A Immediate relief for every patient | B A comparison with placebo |
| C Assessment at week 12 | D A defined adult study population |

2. What remains necessary before external use?

- | | |
|---|--|
| A Reuse of approval from any earlier version | B Review and approval of the complete revised item through the stated process |
| C Only a check that the headline uses familiar words | D Only the brand colleague's confirmation of the deadline |

3. Why is the risk footnote a concern?

- | | |
|---|---|
| A Its unreadable presentation undermines meaningful communication of risk. | B Any footnote is automatically prohibited in every context. |
| C Risks cease to matter when the benefit is a mean difference. | D Risk information belongs only in internal records. |

Answers and explanations: [page 88](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

approved labeling

The regulator-authorized product information in the relevant jurisdiction.

IN USE check approved labeling

prescribing information

The professional product information describing authorized use and relevant details.

IN USE consult the prescribing information

promotional claim

A statement or implication used to communicate a product benefit or attribute in promotion.

IN USE substantiate a promotional claim

claim substantiation

Evidence supporting the precise meaning of a claim.

IN USE verify claim substantiation

fair balance

Appropriate presentation of benefit and risk information in the relevant promotional context.

IN USE assess fair balance

risk disclosure

Communication of relevant risks and limitations.

IN USE improve risk disclosure

material limitation

A qualification important to understanding a claim accurately.

IN USE retain material limitations

intended audience

The people a communication is designed to reach.

IN USE identify the intended audience

indication boundary

The limits of the use or population described by the relevant authorization.

IN USE preserve the indication boundary

onset claim

A claim about when a treatment effect begins.

IN USE substantiate an onset claim

comparative claim

A claim comparing products, treatments, or outcomes.

IN USE review comparative claims

absolute claim

An unqualified assertion suggesting no exceptions.

IN USE challenge an absolute claim

Precision check

A group mean difference does not mean every patient benefits. A week-12 assessment does not establish immediate onset. Qualifiers about population, comparator, timing, and measure carry substantive meaning; removing them can create a different claim even when individual words remain familiar.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

net impression

The overall meaning conveyed by words, visuals, and presentation together.

IN USE evaluate the net impression

qualifier

Language restricting the scope of a statement.

IN USE keep the qualifier visible

medical affairs

The function supporting scientific and medical exchange within its responsibilities.

IN USE consult medical affairs

medical information

The function providing appropriate responses to product-related medical inquiries.

IN USE route a medical-information inquiry

unsolicited request

An inquiry not prompted or solicited through the relevant promotional activity.

IN USE document an unsolicited request

off-label use

Use outside the authorized labeling in the relevant jurisdiction.

IN USE distinguish an off-label question

scientific exchange

Communication of scientific information in an appropriate context.

IN USE define the scientific-exchange context

medical-legal-regulatory review

Cross-functional review of materials under the applicable company process.

IN USE complete MLR review

reference pack

The supporting evidence linked to a communication's claims.

IN USE update the reference pack

redline

A version showing proposed text changes.

IN USE review the redline

approval expiry

The end of a material's permitted use period under the applicable process.

IN USE check approval expiry

final-form approval

Authorization of the complete version in its intended presentation.

IN USE obtain final-form approval

Precision check

MLR means medical-legal-regulatory review; USPI means US prescribing information. Review must consider context, audience, channel, visuals, and risks. These examples are not approved copy; follow applicable requirements and the actual review process.

Phrases you can actually use

C Notice the language / 1 of 2

IDENTIFY THE OVERREACH

"Immediate relief is not supported by the week-12 assessment."

KEEP THE MEASURE

"The finding is a difference in mean symptom score, not universal relief."

CHALLENGE AN ABSOLUTE

"Every patient implies a guarantee the source does not establish."

MAKE RISK READABLE

"Risk information must not be reduced to unreadable text."

PRESERVE THE POPULATION

"The supplied evidence concerns the defined adult study population."

NAME THE COMPARATOR

"Retain versus placebo where it is part of the supported statement."

REVIEW THE WHOLE ITEM

"The headline, visual, and footnote must be assessed together."

TIE THE COMMENT TO EVIDENCE

"Please align this sentence with the cited source and its limitations."

07

Mean versus every

A group average does not describe every individual's response.

At week 12 versus immediately

These are different timing claims.

Overall presentation

A technically accurate sentence can still sit within a misleading visual message.

Phrases you can actually use

C Notice the language / 2 of 2

ROUTE THE INQUIRY

"The off-label question should follow the appropriate medical-information process."

CHECK THE VERSION

"Approval of an earlier item does not establish approval of this revision."

PRESERVE THE CAVEAT

"This qualifier belongs next to the claim it limits."

CLOSE THE ACTION

"I will identify the unsupported implication and the supported alternative."

SEPARATE CONTEXTS

"A scientific response is not automatic permission to reuse a claim in promotion."

REQUEST FINAL REVIEW

"Return the complete final-form item through the required review route."

AVOID DEADLINE PRESSURE

"The publication date does not replace approval."

STATE CURRENT STATUS

"The item remains pending review and is not ready for external use."

Tiny qualification

A qualification that cannot be read does not reliably correct the headline's impression.

Off-label inquiry

Route through the appropriate process; do not improvise a promotional response.

Final form

Review the version that will actually be used, including layout and linked content.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which revision best preserves the supplied scope?

- A** Immediate improvement in the average patient
- B** Proven symptom relief for all adults who receive treatment
- C** Lower mean symptom score versus placebo at week 12 in the defined adult study population
- D** Relief in adults, with timing omitted because the study lasted twelve weeks

2. What is the best review comment on the headline?

- A** It is acceptable once a source number is added.
- B** It is only a stylistic change because relief and score are related.
- C** It is acceptable if the universal claim is placed in a visual instead of text.
- D** It adds an onset claim and an individual guarantee absent from the supplied evidence.

3. Which action addresses the footnote problem?

- A** Remove the risk information to reduce visual clutter.
- B** Assume the source document's risks need not appear in a reviewed communication.
- C** Revise the risk presentation so it is readable and assess the complete item's balance.
- D** Keep the text unchanged because it is technically present.

4. A colleague asks about an unapproved use. What is the appropriate response here?

- A** Add the use to the promotional slide while review is pending.
- B** Treat an internal discussion as a new approved indication.
- C** Answer with the strongest benefit sentence from the draft.
- D** Route the inquiry through the relevant medical-information process without repurposing it as promotion.

Answers and explanations: [page 88](#).

Stronger wording changes the claim

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

final-form approval / medical-legal-regulatory review / absolute claim / material limitation / onset claim / medical information / net impression / fair balance / reference pack / promotional claim

07

- 01 **Adam:** The creative team shortened the headline to immediate relief for every patient. They say it carries the same idea as the approved source, but more clearly.
- 02 **Sofia:** It changes the ¹ _____. The source describes a lower mean symptom score versus placebo at week twelve in a defined adult population. Those qualifications are part of the meaning.
- 03 **Adam:** I see that every patient is stronger than an average difference. The study result does not tell us that all participants responded in the same way.
- 04 **Sofia:** Correct. That ² _____ is unsupported. We should not turn a group result into an individual guarantee, even if the wording sounds accessible and avoids technical language.
- 05 **Adam:** Immediate is also new. The source identifies a week-twelve assessment, but it does not say when any effect began.
- 06 **Sofia:** Then remove the ³ _____ unless appropriate supporting evidence and review establish it. A timepoint of measurement cannot silently become a claim about the first moment of benefit.
- 07 **Adam:** Could we retain a shorter line and put all the conditions in the footnote? The current layout has very little space below the image.
- 08 **Sofia:** A ⁴ _____ must remain meaningfully available in context. Moving essential conditions into unreadable text does not reliably repair the stronger message in the headline.
- 09 **Adam:** The risk information has the same problem. It is technically on the page, but at the current size it is difficult to read.
- 10 **Sofia:** Review the ⁵ _____ of the complete item. We need to assess how benefit and risk are communicated, not just tick a box saying that a block of risk text exists.

The conversation, continued

E Conversations / turns 11-20

07

Continue with the word bank on page 64.

- 11 **Adam:** The image shows a person returning immediately to normal activity. Even with a revised sentence, that could keep suggesting a rapid and universal benefit.
- 12 **Sofia:** Exactly. Assess the ⁶ _____ created by words and visuals together. Correcting one sentence is insufficient if the overall presentation still makes the unsupported implication.
- 13 **Adam:** I will send the revised wording with the evidence references. That should make it easier to see which source supports each part of the claim.
- 14 **Sofia:** Update the ⁷ _____ and check the exact scope. A source citation is useful only when it supports the statement actually made, including population, comparator, timing, and outcome.
- 15 **Adam:** A field colleague has also asked whether this could be used for a different, unapproved condition. That question should not go into this promotional revision.
- 16 **Sofia:** Route it through ⁸ _____ under the applicable process. The existence of a scientific question does not expand the approved indication or authorize a new promotional use.
- 17 **Adam:** Once the edits are complete, can the brand team publish because the original item was approved? The deadline is close, and most of the layout is unchanged.
- 18 **Sofia:** The revised item still needs ⁹ _____. Earlier approval does not automatically cover changed claims or presentation. The deadline does not replace the required decision.
- 19 **Adam:** I will keep it pending, attach the redline, and return the full item through review. The revised wording will match the supplied evidence.
- 20 **Sofia:** Good. The ¹⁰ _____ should see the actual version intended for use. We can make the message readable without deleting the limits that make it accurate.

ANSWERS / EXPLANATIONS: PAGE 89

1. promotional claim | 2. absolute claim | 3. onset claim | 4. material limitation | 5. fair balance | 6. net impression | 7. reference pack | 8. medical information | 9. final-form approval | 10. medical-legal-regulatory review

Earlier approval, changed meaning

F Say it

An approved internal example says benefit was measured at week 12 in adults. A revision says rapid benefit in all ages. The revised item has not completed review.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

population / version / pending / onset

Reviewer: "Rapid adds an unsupported ¹ _____ claim."

Author: "All ages expands the stated ² _____."

Reviewer: "Earlier approval does not cover this changed ³ _____."

Author: "External use remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 89

1. onset | 2. population | 3. version | 4. pending

Rehearse with a fixed script

1. Check your answers, then read turns 1-10 with a partner using the corrected words.
2. Switch roles for turns 11-20. Stress words that distinguish facts from conditions or estimates.
3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Two sources, different questions

A Read the situation

Explain differences between study designs and keep evidence, economic assumptions, and access decisions distinct.

An access deck combines a 12-week randomized placebo-controlled trial in 400 adults with a six-month observational analysis of routine-care claims from 5,000 adults. The trial measures symptom score; the claims analysis measures hospitalization. Baseline differences and treatment-selection factors remain incompletely assessed in the claims analysis. The draft calls the combined material one randomized proof of reduced hospitalizations and guaranteed coverage. Access lead Mina and evidence specialist Jules must separate the evidence streams and qualify both causal and payer claims. No payer decision has been received.

PEOPLE / Mina: Market-access lead | Jules: Evidence specialist

08

DIFFERENT EVIDENCE CAN COMPLEMENT WITHOUT MERGING

Explain what each source answers before placing results side by side. A larger routine-care dataset may broaden context without becoming randomized evidence. Likewise, clinical results, economic models, and coverage decisions address different questions. A useful access conversation makes these boundaries visible without dismissing a source simply because it has limitations.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which source measures hospitalization?

A The 12-week trial's stated symptom-score endpoint

C A completed payer coverage decision

B Both sources using an identical protocol

D The six-month observational claims analysis

2. Why is one randomized proof inaccurate?

A Routine-care data can never contribute evidence.

C Placebo-controlled studies cannot measure symptoms.

B A 5,000-person analysis automatically has fewer participants than a 400-person trial.

D The claims analysis is observational and uses a different outcome and follow-up period.

3. What is the status of coverage?

A Identical to the investigator's efficacy assessment

C Guaranteed by the observational sample size

B No payer decision has been received.

D Approved by the trial's completion

Answers and explanations: page 90.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

market access

Work addressing whether and how patients can obtain a product within a health system.

IN USE develop the market-access plan

payer

An organization or body funding or reimbursing health care.

IN USE address payer evidence needs

coverage decision

A decision defining whether and under what conditions a payer funds a service or product.

IN USE track the coverage decision

formulary

A payer or system's list of medicines and associated access conditions.

IN USE review formulary status

prior authorization

A requirement for advance approval before specified coverage applies.

IN USE clarify prior-authorization criteria

health technology assessment

Structured evaluation of a health technology's effects and implications in context.

IN USE prepare for health technology assessment

value dossier

A structured presentation of clinical, economic, and other evidence for an access audience.

IN USE tailor the value dossier

real-world data

Routinely collected information about health or health-care delivery.

IN USE assess real-world data quality

real-world evidence

Clinical evidence generated by analyzing real-world data using an appropriate study design.

IN USE evaluate real-world evidence

claims data

Records created for billing or reimbursement of health-care services.

IN USE assess claims-data limitations

observational study

A study in which the investigator does not assign the treatment under study.

IN USE describe the observational study

confounding

Distortion of an exposure-outcome comparison by other relevant factors.

IN USE address confounding

Precision check

Real-world describes data context, not guaranteed quality or one study design. Randomized studies can use routine data. This claims comparison, however, is observational, with treatment-selection issues still incompletely assessed.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

confounding by indication

Differences in treatment choice related to prognosis or clinical need that affect comparison.

IN USE assess confounding by indication

selection bias

Distortion related to how participants or observations enter the analysis.

IN USE examine selection bias

residual confounding

Confounding remaining after the applied adjustment.

IN USE acknowledge residual confounding

external validity

How well findings apply beyond the studied setting or population.

IN USE assess external validity

comparative effectiveness

Comparison of health outcomes from different care options in relevant settings.

IN USE evaluate comparative effectiveness

budget impact

The expected change in spending for a specified budget holder over a defined period.

IN USE model budget impact

cost-effectiveness

Assessment of costs relative to health outcomes for compared options.

IN USE examine cost-effectiveness assumptions

uptake assumption

An estimate of how many eligible people will use a product over time.

IN USE test the uptake assumption

launch readiness

Preparedness of the relevant functions for a planned market introduction.

IN USE verify launch readiness

biosimilar

A biological product shown to be highly similar to a reference product without clinically meaningful safety or effectiveness differences.

IN USE evaluate a biosimilar evidence package

reference product

The authorized biological product used as the comparator for biosimilar development.

IN USE identify the reference product

lifecycle management

Planned development and stewardship of a product across its commercial and regulatory life.

IN USE align lifecycle management

Precision check

RWD means real-world data; RWE, real-world evidence; HTA, health technology assessment. Budget impact estimates spending; cost-effectiveness relates costs to health outcomes. Neither analysis, nor regulatory approval alone, establishes payer coverage.

Phrases you can actually use

C Notice the language / 1 of 2

SEPARATE THE SOURCES

"The trial and claims analysis answer different questions."

KEEP THE OUTCOME CLEAR

"The trial measures symptom score; the claims analysis measures hospitalization."

QUALIFY CAUSALITY

"Treatment-selection differences may affect the observed association."

AVOID SAMPLE-SIZE OVERREACH

"A larger dataset does not remove systematic bias by itself."

STATE THE DESIGN

"The claims comparison is observational, not randomized."

PRESERVE FOLLOW-UP

"The studies use different observation periods."

ASK ABOUT ADJUSTMENT

"Which confounders were measured, and what uncertainty remains?"

RETAIN USEFUL CONTEXT

"Routine-care evidence may add context without replacing the trial's design."

08

Association versus causation

An observed difference needs a defensible causal design and assumptions before a causal interpretation.

Large versus unbiased

More records can increase precision without removing confounding.

Trial versus routine care

State the actual design instead of ranking evidence by a label alone.

Phrases you can actually use

C Notice the language / 2 of 2

SEPARATE ECONOMIC QUESTIONS

"Budget impact and cost-effectiveness are not interchangeable."

LIMIT THE COVERAGE CLAIM

"No payer decision has been received."

DISTINGUISH SIMILARITY

"Biosimilarity is a specific evidence and regulatory concept, not a loose claim of being similar."

TRACK LAUNCH DEPENDENCIES

"Clinical evidence does not complete every access and supply dependency."

EXPOSE MODEL INPUTS

"Uptake and eligible-population assumptions should remain visible."

CHECK THE AUDIENCE

"Which evidence question is this payer asking us to address?"

AVOID AUTOMATIC SUBSTITUTION CLAIMS

"Substitution implications require the relevant product status and applicable rules."

CLOSE THE DECK

"Label each evidence stream, its limitations, and the decision it can inform."

08

Coverage versus approval

Regulatory authorization and payer funding are different decisions.

Biosimilar versus interchangeable

These are not identical terms; implications depend on jurisdiction and product status.

Forecast versus commitment

A modeled uptake rate is not a payer commitment or confirmed patient count.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which summary is best supported?

- A** The claims analysis is randomized because the trial is randomized.
- B** The larger sample makes treatment-selection assessment unnecessary.
- C** The trial assesses symptoms; the observational claims study examines hospitalizations with unresolved confounding questions.
- D** Both studies independently prove the same hospitalization effect.

2. What should be checked before interpreting the claims difference causally?

- A** Whether the trial used an unrelated symptom scale
- B** Treatment selection, measured confounders, remaining bias, and the analysis design
- C** Only whether the sample exceeds the trial's sample
- D** Only whether the result supports the launch message

3. Which statement about economic evidence is accurate?

- A** An uptake assumption is the same as an executed coverage decision.
- B** Budget impact estimates spending for a defined budget holder; cost-effectiveness relates costs to health outcomes.
- C** Budget impact proves that a product is clinically superior.
- D** Cost-effectiveness guarantees that every payer will provide coverage.

4. Which launch update preserves the actual status?

- A** The access evidence package is being revised; payer decisions remain pending.
- B** Coverage is guaranteed because the evidence sources were combined.
- C** Every launch dependency is complete once a trial ends.
- D** The claims dataset authorizes unrestricted substitution of all biological products.

Answers and explanations: page 90.

Two sources, different questions

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Residual confounding / launch-readiness / observational design / uptake assumption / evidence streams / coverage decision / external validity / Real-world data / confounding by indication / budget impact

08

- 01 **Mina:** The access deck combines the trial and claims results under one headline: randomized proof of fewer hospitalizations and guaranteed coverage. That seems to merge several different claims.
- 02 **Jules:** Start by separating the ¹ _____. The trial assessed symptom score over twelve weeks. The claims analysis assessed hospitalization over six months in a different, routine-care population.
- 03 **Mina:** The trial randomized four hundred adults, while the claims dataset contains five thousand. The larger number is attracting more attention in the discussion.
- 04 **Jules:** Size does not change the ² _____. Treatment was not assigned by the claims study. We need to examine how people entered each treatment group and which differences might affect their outcomes.
- 05 **Mina:** Patients receiving one treatment could have had different clinical needs before follow-up began. Those needs might also affect their chances of hospitalization.
- 06 **Jules:** That raises ³ _____. We should ask which relevant factors were measured and addressed, while acknowledging that the current assessment is incomplete.
- 07 **Mina:** The team adjusted for some recorded characteristics. That is useful, but the slide now says adjustment removed every possible source of bias.
- 08 **Jules:** It cannot establish that from the supplied information. ⁴ _____ may remain, particularly for factors not adequately captured. Explain the approach and limitations rather than describe adjustment as a universal cure.
- 09 **Mina:** We should still show what the routine-care analysis adds. It may help an access audience understand patterns outside the trial's narrower setting.
- 10 **Jules:** Yes. Discuss ⁵ _____ without assuming that all routine-care records represent every intended patient. Data relevance and quality need examination alongside the study design.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Mina:** The phrase real-world evidence is being used as though it automatically means observational. That is too broad, even though this particular study is observational.
- 12 **Jules:** Correct. ⁶ _____ describes the data context. Different designs can use those data. In this case, we should state the actual observational design rather than let the label do the explaining.
- 13 **Mina:** The financial slide also combines budget impact with cost-effectiveness. One estimates payer spending, while the other relates costs to health outcomes.
- 14 **Jules:** Keep the ⁷ _____ model separate and state its horizon, eligible population, and uptake assumptions. A useful economic result depends on a clearly defined question and inputs.
- 15 **Mina:** The uptake assumption is twenty percent of an estimated eligible population. It is a planning input, not a confirmed number of people who will receive coverage.
- 16 **Jules:** Then label the ⁸ _____ prominently. The model should not transform an estimate into a commitment simply because it appears in a launch forecast.
- 17 **Mina:** We have no payer decision yet. The headline's guaranteed coverage wording could create expectations that neither the evidence team nor the access team can support.
- 18 **Jules:** A ⁹ _____ is distinct from the clinical findings and economic models. State the current status and any unresolved evidence requests instead of promising an outcome.
- 19 **Mina:** I will revise the deck into separate trial, routine-care, economic, and access sections. Each result will retain its design, population, period, and limitations.
- 20 **Jules:** That supports a realistic ¹⁰ _____ discussion. We can explain what is ready, what remains under review, and who owns each dependency without presenting a collection of evidence as a completed launch decision.

08

ANSWERS / EXPLANATIONS: PAGE 91

1. evidence streams | 2. observational design | 3. confounding by indication | 4. Residual confounding | 5. external validity | 6. Real-world data | 7. budget impact | 8. uptake assumption | 9. coverage decision | 10. launch-readiness

An assumption is not a covered population

F Say it

A fictional spending model assumes 10,000 eligible adults, 20% uptake, and \$300 incremental annual spending per user. No offsets or other costs are included. No payer decision is available.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

assumption / undecided / 2,000 / \$600,000

Analyst: "The model assumes ¹ _____ users."

Reviewer: "The modeled annual spending increase is ² _____."

Analyst: "The 20% figure is an uptake ³ _____."

Reviewer: "Coverage remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 91

1. 2,000 | 2. \$600,000 | 3. assumption | 4. undecided

Rehearse with a fixed script

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3. Complete and check the four-line exchange. Read it twice, switching roles and keeping the printed facts.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

01 Briefing and language checks

Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

A Briefing checks / page 4

1. Where was the observed result obtained?

A. In a cell-based assay across three experimental runs

The supplied evidence is laboratory evidence, with no patient study or approved label in the case.

2. What does the target product profile represent here?

A. Desired future characteristics to guide development

The proposed profile sets aspirations; it does not establish that those characteristics have been demonstrated.

3. Which claim exceeds the evidence?

D. PX-17 improves patient mobility.

No patients were studied, so the laboratory result cannot establish a patient mobility benefit.

D Language checks / page 9

1. Which replacement is best supported?

C. PX-17 changed the marker in the tested cell assay; patient benefit remains unestablished.

The replacement preserves the observed laboratory finding while explicitly limiting the clinical inference.

2. What does testing one concentration fail to characterize?

A. How the effect changes across different concentrations

A response across concentrations requires additional concentration conditions; the supplied experiment used only one.

3. Which question best tests the evidence bridge?

A. How well does this marker predict the intended patient outcome in this context?

The question asks for the missing connection between the measured marker and the claimed patient benefit.

4. What can the portfolio discussion decide on the supplied authority?

C. A proposed next investigation for appropriate review, not automatic clinical-study authorization

The brief excludes clinical-study authorization and supplies no basis for treatment or marketing claims.

01 Conversation answers

Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

E Dialogue gaps

Pages 10-11

- 1. cell-based assay.** Turn 2. A cell-based assay is laboratory evidence, not a study establishing outcomes in patients.
- 2. biomarker.** Turn 4. The biomarker is the measured biological characteristic; its change does not itself establish improved patient function.
- 3. target product profile.** Turn 6. The target product profile sets desired future characteristics rather than recording benefits already demonstrated.
- 4. clinical benefit.** Turn 8. Clinical benefit concerns meaningful patient outcomes; this experiment did not measure any patient outcome.
- 5. dose-response relationship.** Turn 10. Dose-response describes effects across doses; the single tested concentration cannot establish that relationship.
- 6. reproducibility.** Turn 12. Reproducibility concerns consistent findings under specified repeat conditions, not automatic generalization to humans.
- 7. tolerability.** Turn 14. Tolerability concerns enduring unwanted effects in a defined context; this laboratory experiment did not establish human tolerability.
- 8. evidence gap.** Turn 16. The evidence gap is missing information connecting the assay finding to a more meaningful development conclusion.
- 9. go/no-go criterion.** Turn 18. A go/no-go criterion identifies evidence needed for a development decision rather than treating all additional data as progress.
- 10. development milestone.** Turn 20. A development milestone is a defined progress or review point, not automatic authorization for human studies.

F Transfer conversation

Page 12

1. goal

Scientist: "Once-daily use is currently a goal."

The planning profile states an intended characteristic, while no human dosing evidence is supplied.

2. target engagement

Reviewer: "The measured laboratory finding concerns target engagement."

The test measured interaction with the intended target, not patient symptoms.

3. unestablished

Scientist: "Human dosing remains unestablished."

The brief explicitly says there are no human dosing data.

4. improvement

Reviewer: "We cannot claim symptom improvement."

Patient symptoms were not measured, so an improvement claim would exceed the evidence.

02 Briefing and language checks

Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

A Briefing checks / page 13

1. What is wrong with the current question?

C. It asks broadly about the entire plan instead of the specific unresolved proposal.

The real issue concerns population and endpoint rationale, while the draft requests undifferentiated agreement with the entire plan.

2. What is the status of two supporting reports?

B. Unfinished

The brief explicitly says two supporting reports remain unfinished.

3. Which statement is accurate now?

B. No agency response has been received.

Preparation is ongoing, and the case supplies no response or study authorization.

D Language checks / page 18

1. Which revised question is most useful?

B. Does the agency agree with the proposed population and endpoint rationale described in section three?

The focused question identifies the proposal and its supporting section without seeking blanket approval.

2. Which package statement is accurate?

D. Two reports remain unfinished and their status is identified.

The statement preserves the actual document status and allows reviewers to understand the information gap.

3. Which distinction is correct in US terminology?

D. An IND supports clinical investigation; an NDA seeks marketing approval for a new drug.

These submissions serve different purposes, and permission or advice at one stage does not guarantee later approval.

4. How should limited advice be summarized?

D. Preserve its subject, assumptions, conditions, and unresolved issues.

An accurate record retains the boundaries of the actual response instead of changing its meaning.

02 Conversation answers

Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

E Dialogue gaps

Pages 19-20

- 1. focused question.** Turn 2. A focused question identifies the specific unresolved issue, here population and endpoint rationale.
- 2. sponsor position.** Turn 4. The sponsor position is the company's proposed approach, which must not be confused with agency agreement.
- 3. information gap.** Turn 6. The information gap is the unfinished supporting material, whose status must remain visible in the package.
- 4. briefing document.** Turn 8. The briefing document should present a coherent proposal, evidence, and questions rather than conflicting population descriptions.
- 5. cross-reference.** Turn 10. A cross-reference must lead to material that actually supports the statement, including after document revisions.
- 6. investigational new drug application.** Turn 12. An investigational new drug application concerns clinical investigation in the US; a meeting package is not itself that authorization.
- 7. regulatory pathway.** Turn 14. The regulatory pathway is the applicable route for this product, which qualified regulatory colleagues must confirm.
- 8. conditional advice.** Turn 16. Conditional advice applies within stated circumstances; removing those conditions changes the meaning of the response.
- 9. meeting minutes.** Turn 18. Meeting minutes record the discussion and outcomes; discrepancies should be resolved through the applicable process.
- 10. approval status.** Turn 20. Approval status must reflect the actual authorization record, not expectations created by meeting preparation or positive discussion.

F Transfer conversation

Page 21

1. conditional

Regulatory lead: "The endpoint advice is conditional."

The note limits support to the population actually described, so it is not unconditional agreement.

2. unresolved

Clinical lead: "Follow-up duration remains unresolved."

Further justification is requested, so the note does not settle that issue.

3. population

Regulatory lead: "Preserve the stated population."

The population defines the scope within which the endpoint advice applies.

4. authorization

Clinical lead: "This is not marketing authorization."

The briefing explicitly excludes marketing authorization, regardless of the favorable endpoint comment.

03 Briefing and language checks

Clinical Trial Design, Protocols, GCP, and Operations

A Briefing checks / page 22

1. What is the documented conflict?

D. Three-day versus seven-day visit windows in two sources

The brief supplies conflicting window descriptions and no confirmed controlling version.

2. What has not been confirmed?

C. The controlling version and its implementation status at the site

Both document authority and site applicability remain unresolved in the supplied facts.

3. Which response avoids an unauthorized assumption?

B. Escalate the exact discrepancy for documented clarification through the applicable process.

The case requires authorized clarification, not a convenient choice, inferred authority, or silent alteration of records.

D Language checks / page 27

1. Which clarification request is strongest?

B. Please confirm the approved site-effective version and section governing the visit window.

The request asks for authority and applicability rather than convenience or an invented compromise.

2. What does a newer file date prove by itself?

C. Only that the file carries a later date, not that it governs the site

File recency alone does not establish approval, effective status, training, or authority to change records.

3. How should a necessary record correction be handled?

A. Through the applicable traceable correction process, preserving the original history

Traceable correction preserves what was recorded and why it changed, instead of concealing the original entry.

4. What should follow an authorized clarification?

C. Confirm understanding and assess affected appointments or records through the appropriate process.

The clarified instruction must reach the relevant people, and any impact needs assessment rather than an unsupported conclusion.

03 Conversation answers

Clinical Trial Design, Protocols, GCP, and Operations

03

E Dialogue gaps

Pages 28-29

1. controlling version. Turn 2. The controlling version is the authorized document applicable to this site, not whichever file is easier to use.

2. effective date. Turn 4. The effective date establishes when a document applies; a later email alone does not establish implementation status.

3. schedule of assessments. Turn 6. The schedule of assessments supplies timing details that must be reconciled with the governing protocol.

4. principal investigator. Turn 8. The principal investigator leads the study at the site and is part of the appropriate clarification route.

5. protocol amendment. Turn 10. A protocol amendment changes the study document and requires applicable authorization and implementation, not merely receipt by email.

6. training record. Turn 12. The training record helps establish whether the site team received the required instruction for an implemented change.

7. protocol deviation. Turn 14. A protocol deviation is a departure from the protocol; classification requires knowing which requirement actually applied.

8. audit trail. Turn 16. The audit trail preserves the history of recorded changes; silently altering dates would conceal rather than resolve the discrepancy.

9. read-back. Turn 18. A read-back repeats the key instruction and source, allowing misunderstanding to be detected before the team acts.

10. essential records. Turn 20. Essential records support reconstruction and evaluation of trial conduct, including the documented clarification relevant to this discrepancy.

F Transfer conversation

Page 30

1. received

Coordinator: "Version 5 has been received."

Email receipt is established, but the briefing supplies no confirmation of authorized implementation.

2. unconfirmed

Monitor: "Its site applicability remains unconfirmed."

The available record still names version 4, so applicability must be clarified.

3. confirmation

Coordinator: "I will request implementation confirmation."

The missing information concerns whether and when the newer version applies at the site.

4. recency

Monitor: "Do not infer authority from file recency."

A newer file alone does not establish the approval and effective status required to govern the work.

04 Briefing and language checks

Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

A Briefing checks / page 31

1. Did the primary analysis meet the specified significance threshold?

A. No; $p=0.19$ is above 0.05.

The supplied p-value exceeds the prespecified threshold, and the interval actually includes both negative and positive values.

2. What is the secondary endpoint's status under this plan?

B. A nominal result that cannot support the stated confirmatory claim after primary failure

The specified testing sequence requires primary success before a confirmatory secondary claim.

3. What does the negative point estimate indicate?

C. The estimated difference favors treatment on this lower-is-better scale.

Treatment minus control is negative on a scale where lower is better, but uncertainty and the test result still matter.

D Language checks / page 36

1. Which headline is most accurate?

D. Primary threshold not met; secondary finding is nominal under the prespecified plan.

The accurate headline preserves both the primary outcome and the limited status of the secondary result.

2. What does the interval from -5 to +1 show?

A. It includes values favoring treatment, zero, and values favoring control on this scale.

The interval crosses zero and spans both directions of the treatment-minus-control difference.

3. Why does clinical importance need separate discussion?

C. The practical meaning of a score difference is not established by a p-value alone.

Statistical evidence alone does not determine whether an effect matters to patients in the relevant clinical context.

4. Which statement about later subgroup work is appropriate?

B. Label it according to its actual prespecification and multiplicity status.

Transparency about timing and testing status prevents exploratory work from being misrepresented as confirmatory evidence.

04 Conversation answers

Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

E Dialogue gaps

Pages 37-38

- 1. primary endpoint.** Turn 2. The primary endpoint addresses the principal prespecified objective and cannot be replaced by a favorable secondary finding.
- 2. point estimate.** Turn 4. The point estimate is the estimated two-point difference favoring treatment; it must be reported with uncertainty.
- 3. confidence interval.** Turn 6. The confidence interval crosses zero and includes differences in both directions, showing uncertainty around the estimate.
- 4. testing hierarchy.** Turn 8. The testing hierarchy conditions the confirmatory secondary claim on primary success, which the supplied result did not achieve.
- 5. nominal p-value.** Turn 10. The nominal p-value can be reported with its limitation, but lacks the confirmatory status required by this testing plan.
- 6. Clinical importance.** Turn 12. Clinical importance concerns the meaning of the effect for patients, which a p-value alone cannot establish.
- 7. estimand.** Turn 14. The estimand defines the treatment effect of interest, including how relevant intercurrent events are handled.
- 8. intercurrent event.** Turn 16. An intercurrent event occurs after treatment starts and affects the interpretation or existence of relevant measurements.
- 9. sensitivity analyses.** Turn 18. Sensitivity analyses examine robustness to assumptions for the target question, rather than searching only for a favorable result.
- 10. topline readout.** Turn 20. The topline readout should summarize the key results with their uncertainty and actual testing status.

F Transfer conversation

Page 39

1. 20%

Analyst: "The control event rate is 20%."

Dividing 200 events by 1,000 participants gives 0.20, or 20%.

2. 15%

Reviewer: "The treatment event rate is 15%."

Dividing 150 events by 1,000 participants gives 0.15, or 15%.

3. five percentage points

Analyst: "The absolute reduction is five percentage points."

Subtract 15% from 20%; the difference between the rates is five percentage points.

4. 25%

Reviewer: "Relative to control, the reduction is 25%."

The five-percentage-point reduction divided by the 20% control rate equals 25%; this does not establish statistical significance.

05 Briefing and language checks

Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

A Briefing checks / page 40

1. What has the clinician actually reported?

A. Dizziness described as severe after product use

The message reports dizziness and its described intensity, while key assessment details remain missing.

2. Can severe automatically be treated as serious in the regulatory sense?

D. No; intensity and seriousness criteria are different concepts.

Severe describes intensity, whereas seriousness depends on applicable outcome or medical criteria that are not confirmed here.

3. What does the stated company procedure require?

D. Prompt internal routing of available information with documented follow-up

The procedure requires routing despite incompleteness and does not authorize diagnosis or treatment advice.

D Language checks / page 45

1. Which handoff is most precise?

C. Severe dizziness was reported after LM-4 use; seriousness and causality remain unassessed.

The precise handoff preserves the reported symptom while distinguishing unknown classifications and outcomes.

2. What should happen while missing details are sought?

C. Route available information promptly under the stated procedure and document follow-up.

The stated process requires timely internal routing and follow-up, not waiting for complete information or providing treatment advice.

3. Which follow-up asks for a missing fact without leading the reporter?

D. Was the patient hospitalized, and what was the outcome?

The neutral question seeks hospitalization and outcome facts that are absent without suggesting the desired answer.

4. What does a temporal association establish?

A. That the event and product use have a stated timing relationship, not necessarily causation

Temporal association concerns timing; causal conclusions and labeling decisions require the relevant assessment.

05 Conversation answers

Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

E Dialogue gaps

Pages 46-47

- 1. case intake.** Turn 2. Case intake records the available report so assessment and follow-up can proceed under the required process.
- 2. severity.** Turn 4. Severity describes the reported intensity, while seriousness depends on separate applicable criteria.
- 3. temporal association.** Turn 6. Temporal association concerns timing and does not by itself prove that the product caused the event.
- 4. concomitant medications.** Turn 8. Concomitant medications are other medicines used in the relevant period and are missing from this report.
- 5. verbatim term.** Turn 10. The verbatim term preserves the original reporter's wording, even when standardized coding is added later.
- 6. follow-up request.** Turn 12. A follow-up request seeks missing facts neutrally rather than suggesting the answer or demanding proof of causation.
- 7. case reconciliation.** Turn 14. Case reconciliation checks related records and handoffs so the same case is consistently accounted for across teams.
- 8. safety signal.** Turn 16. A safety signal suggests a possible new or changed risk requiring evaluation, not a confirmed causal conclusion from this handoff.
- 9. benefit-risk assessment.** Turn 18. Benefit-risk assessment considers favorable and unfavorable effects in context; this incomplete report does not determine a labeling decision.
- 10. receipt confirmation.** Turn 20. Receipt confirmation closes the handoff by establishing that the safety team received the information.

F Transfer conversation

Page 48

1. unknown

Associate: "Hospitalization status is unknown."

The report contains no hospitalization information, so neither occurrence nor absence can be asserted.

2. causation

Reviewer: "The timing does not establish causation."

Occurrence after use supplies a temporal relationship, not proof that the product caused the rash.

3. now

Associate: "I will route the available report now."

The stated process requires routing the information now rather than waiting for every missing detail.

4. follow-up

Reviewer: "Record the missing details for follow-up."

Additional information is needed, and the reporting process must preserve the outstanding questions.

05

06 Briefing and language checks

CMC, CGMP, Quality Events, Manufacturing, and Supply

A Briefing checks / page 49

1. What is established about batch Q-214?

B. A verification entry is missing and quality review remains open.

The brief establishes a documentation gap and open review, not completion, release, or a confirmed manufacturing failure.

2. Why is fully reviewed inaccurate?

C. The required investigation and authorized disposition are not complete.

Laboratory results do not resolve the missing verification or complete the specified quality process.

3. Which action is excluded by the case?

A. Backdating an entry to make the record appear complete

The brief expressly prohibits backdating and requires a documented investigation rather than retrospective fabrication.

D Language checks / page 54

1. Which supply update is supported?

B. Q-214 remains under quality review; release timing is unconfirmed.

The update states the actual open status without assuming either release or a confirmed product defect.

2. What does the missing entry prove by itself?

C. That required recorded information is absent

A missing entry establishes a documentation gap; the underlying event requires investigation.

3. Which action preserves data integrity?

D. Use the authorized correction process with an accurate, traceable account.

An authorized, traceable correction preserves the true record rather than inventing evidence or concealing the gap.

4. What should a preventive response address?

B. The supported cause and a way to check whether recurrence risk is reduced

A meaningful action follows evidence about the cause and includes a check of its intended effect.

06 Conversation answers

CMC, CGMP, Quality Events, Manufacturing, and Supply

E Dialogue gaps

Pages 55-56

1. batch record. Turn 2. The batch record contains a missing verification entry, so the required review is not complete.

2. documentation gap. Turn 4. The documentation gap is the established missing information; its underlying cause has not yet been determined.

3. contemporaneous record. Turn 6. A contemporaneous record is made when the activity occurs; a later entry must not falsely appear to have been made earlier.

4. traceability. Turn 8. Traceability preserves the links between activities, evidence, and changes so the discrepancy can be reconstructed.

5. out-of-specification result. Turn 10. An out-of-specification result is a test result outside acceptance specifications, not merely missing documentation.

6. deviation investigation. Turn 12. The deviation investigation examines the discrepancy through the applicable documented process and identifies supported conclusions.

7. root cause. Turn 14. The root cause must be supported by investigation rather than selected because it is a convenient explanation.

8. effectiveness check. Turn 16. An effectiveness check evaluates whether the action achieved its intended result, rather than merely confirming task completion.

9. batch disposition. Turn 18. Batch disposition is the authorized decision on batch use or handling, which remains pending here.

10. release authorization. Turn 20. Release authorization is formal permission to proceed; it cannot be inferred from available tests or a desired shipment date.

F Transfer conversation

Page 57

1. results

Planner: "The certificate reports test results."

A certificate of analysis reports specified testing information, not every aspect of the release decision.

2. pending

Quality: "Batch disposition is still pending."

The briefing states that the authorized disposition record has not been completed.

3. released

Planner: "The current status is not released."

The supplied status expressly says not released, despite the available certificate.

4. authorization

Quality: "The customer request does not supply release authorization."

A delivery request cannot replace the applicable authorized decision on whether the batch may ship.

07 Briefing and language checks

Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

A Briefing checks / page 58

1. Which part of the draft is unsupported by the supplied source?

A. Immediate relief for every patient

The supplied claim concerns a mean score at week 12, not immediate onset or a benefit for every individual.

2. What remains necessary before external use?

B. Review and approval of the complete revised item through the stated process

The case requires review of the complete revised item, not just a phrase or an earlier version.

3. Why is the risk footnote a concern?

A. Its unreadable presentation undermines meaningful communication of risk.

The problem is the unreadable presentation, not a universal prohibition on footnotes or the existence of a benefit claim.

D Language checks / page 63

1. Which revision best preserves the supplied scope?

C. Lower mean symptom score versus placebo at week 12 in the defined adult study population

Only the first wording retains the measured outcome, comparator, timepoint, and defined population without adding a universal or onset claim.

2. What is the best review comment on the headline?

D. It adds an onset claim and an individual guarantee absent from the supplied evidence.

The draft changes the substantive meaning; a citation or change of presentation does not establish the missing evidence.

3. Which action addresses the footnote problem?

C. Revise the risk presentation so it is readable and assess the complete item's balance.

Meaningful risk communication requires attention to presentation and context, not merely the physical presence of tiny text.

4. A colleague asks about an unapproved use. What is the appropriate response here?

D. Route the inquiry through the relevant medical-information process without repurposing it as promotion.

The communication context and approved scope matter; the inquiry does not authorize an expanded promotional claim.

07 Conversation answers

Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

E Dialogue gaps

Pages 64-65

- 1. promotional claim.** Turn 2. The promotional claim now adds immediate onset and universal benefit, neither of which the supplied evidence supports.
- 2. absolute claim.** Turn 4. An absolute claim suggests no exceptions; a group mean result does not establish benefit for every patient.
- 3. onset claim.** Turn 6. An onset claim concerns when an effect begins; a week-12 assessment does not establish immediate relief.
- 4. material limitation.** Turn 8. A material limitation affects the accurate interpretation of the claim and should not disappear into unreadable text.
- 5. fair balance.** Turn 10. Fair balance concerns appropriate benefit-risk presentation; merely including unreadable risk text does not resolve the issue.
- 6. net impression.** Turn 12. Net impression is the combined meaning of wording, imagery, and layout, not just the literal accuracy of one sentence.
- 7. reference pack.** Turn 14. The reference pack links claims to their supporting evidence and allows reviewers to check the precise scope.
- 8. medical information.** Turn 16. Medical information provides the appropriate route for the inquiry; the question does not authorize an expanded promotional claim.
- 9. final-form approval.** Turn 18. Final-form approval applies to the complete version intended for use; approval of an earlier item does not cover every revision.
- 10. medical-legal-regulatory review.** Turn 20. Medical-legal-regulatory review assesses the revised communication through the required cross-functional process before external use.

F Transfer conversation

Page 66

1. onset

Reviewer: "Rapid adds an unsupported onset claim."

The source supplies a week-12 measurement, not evidence about how quickly benefit begins.

2. population

Author: "All ages expands the stated population."

The original example concerns adults, while the revision extends the claim beyond that group.

3. version

Reviewer: "Earlier approval does not cover this changed version."

A substantive revision must be assessed under the applicable process rather than inheriting approval automatically.

4. pending

Author: "External use remains pending."

The revised item has not completed review, so the briefing supplies no authorization for external use.

08 Briefing and language checks

Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

A Briefing checks / page 67

1. Which source measures hospitalization?

D. The six-month observational claims analysis

The brief assigns hospitalization to the claims analysis and symptom score to the trial.

2. Why is one randomized proof inaccurate?

D. The claims analysis is observational and uses a different outcome and follow-up period.

The sources differ in design, endpoint, and duration; combining slides does not make the observational analysis randomized.

3. What is the status of coverage?

B. No payer decision has been received.

The briefing explicitly says no payer decision exists, so coverage cannot be reported as guaranteed.

D Language checks / page 72

1. Which summary is best supported?

C. The trial assesses symptoms; the observational claims study examines hospitalizations with unresolved confounding questions.

The accurate summary preserves each source's design, outcome, and unresolved limitation.

2. What should be checked before interpreting the claims difference causally?

B. Treatment selection, measured confounders, remaining bias, and the analysis design

Causal interpretation depends on the design and assumptions, not merely sample size or a favorable result.

3. Which statement about economic evidence is accurate?

B. Budget impact estimates spending for a defined budget holder; cost-effectiveness relates costs to health outcomes.

These economic analyses address different questions and do not themselves determine clinical superiority or coverage.

4. Which launch update preserves the actual status?

A. The access evidence package is being revised; payer decisions remain pending.

The brief states that no payer decision has been received and the deck still needs correction.

08 Conversation answers

Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

E Dialogue gaps

Pages 73-74

- 1. evidence streams.** Turn 2. The evidence streams differ in study design, outcome, and follow-up, so their findings cannot be presented as one randomized result.
- 2. observational design.** Turn 4. The observational design did not assign treatment randomly; a larger sample does not change that fact.
- 3. confounding by indication.** Turn 6. Confounding by indication arises when treatment choice relates to clinical need or prognosis that also affects outcomes.
- 4. Residual confounding.** Turn 8. Residual confounding is distortion remaining after adjustment, which cannot be ruled out merely by listing adjusted variables.
- 5. external validity.** Turn 10. External validity concerns applicability beyond the studied setting; routine-care context may help but does not automatically guarantee generalizability.
- 6. Real-world data.** Turn 12. Real-world data describes routinely collected health information; the study design using those data must be specified separately.
- 7. budget impact.** Turn 14. Budget impact estimates spending consequences for a specified budget holder over a defined period, not value per health outcome.
- 8. uptake assumption.** Turn 16. The uptake assumption is a model input about use, not a confirmed patient count or payer commitment.
- 9. coverage decision.** Turn 18. A coverage decision is the payer's funding determination, which has not been received in this case.
- 10. launch-readiness.** Turn 20. Launch-readiness assessment checks multiple dependencies; evidence preparation alone does not establish coverage, supply, or complete operational readiness.

F Transfer conversation

Page 75

1. 2,000

Analyst: "The model assumes 2,000 users."

Twenty percent of 10,000 eligible adults equals 2,000 modeled users, not a confirmed covered population.

2. \$600,000

Reviewer: "The modeled annual spending increase is \$600,000."

Multiply 2,000 modeled users by \$300 each, with no other costs or offsets included.

3. assumption

Analyst: "The 20% figure is an uptake assumption."

The briefing identifies the figure as a planning input rather than an observed or guaranteed use rate.

4. undecided

Reviewer: "Coverage remains undecided."

No payer decision has been supplied, so the model cannot establish that coverage has been granted.

08

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

Locate the evidence: The observation comes from a cell-based assay, not a patient study.

Clarify the aspiration: The target product profile describes what we aim to demonstrate.

State the next question: We need to test whether the finding persists under relevant follow-up conditions.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

Focus the question: We seek advice on the proposed population and endpoint rationale.

Disclose incompleteness: Two supporting reports are not yet final.

Preserve a condition: Record the qualification alongside the advice, not in a separate summary.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Clinical Trial Design, Protocols, GCP, and Operations

State the discrepancy: The protocol attachment says three days; the quick-reference sheet says seven.

Avoid a guess: I cannot confirm the visit window from these conflicting files.

Check training: Has the relevant site team completed the required update?

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

Lead with the primary: The primary analysis did not meet the prespecified significance threshold.

Qualify the secondary: The secondary p-value is nominal under this testing plan.

Define the target: Which treatment effect does the estimand specify?

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

Open the handoff: I have an incomplete report that needs safety intake.

Avoid causality overreach: The event followed use; causality has not been assessed.

Protect the original: Retain the reporter's exact words alongside any coded term.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 CMC, CGMP, Quality Events, Manufacturing, and Supply

State the observed gap: The verification entry is missing from the batch record.

Separate evidence: Available laboratory results do not complete the documentation review.

Limit the supply update: Release timing is unconfirmed pending authorized disposition.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

Identify the overreach: Immediate relief is not supported by the week-12 assessment.

Name the comparator: Retain versus placebo where it is part of the supported statement.

Tie the comment to evidence: Please align this sentence with the cited source and its limitations.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

Separate the sources: The trial and claims analysis answer different questions.

Preserve follow-up: The studies use different observation periods.

Retain useful context: Routine-care evidence may add context without replacing the trial's design.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

absolute claim	challenge an absolute claim	59
adverse event	capture an adverse event	41
adverse reaction	assess a suspected adverse reaction	41
agency interaction	document the agency interaction	14
aggregate safety review	conduct an aggregate safety review	42
analysis population	specify the analysis population	32
approval expiry	check approval expiry	60
approval status	verify approval status	15
approved labeling	check approved labeling	59
assay limitation	disclose assay limitations	6
audit trail	preserve the audit trail	24
batch disposition	document batch disposition	51
batch record	review the batch record	50
benefit-risk assessment	update the benefit-risk assessment	42
biological plausibility	assess biological plausibility	5
biologics license application	prepare the BLA strategy	14
biomarker	evaluate a candidate biomarker	5
biosimilar	evaluate a biosimilar evidence package	69
blinding	maintain blinding	24
briefing document	finalize the briefing document	14
budget impact	model budget impact	69
CAPA	verify CAPA effectiveness	51
case intake	complete case intake	41
case reconciliation	perform case reconciliation	42

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

causality assessment	document causality assessment	41
cell-based assay	qualify the cell-based assay	5
certificate of analysis	verify the certificate of analysis	51
change control	route a change through change control	51
chemistry, manufacturing, and controls	assess CMC readiness	50
claim substantiation	verify claim substantiation	59
claims data	assess claims-data limitations	68
clinical benefit	establish clinical benefit	5
clinical hold	address a clinical hold	15
clinical importance	assess clinical importance	33
clinical research associate	contact the clinical research associate	23
comparative claim	review comparative claims	59
comparative effectiveness	evaluate comparative effectiveness	69
concomitant medication	document concomitant medications	42
conditional advice	preserve conditional advice	15
confidence interval	report the confidence interval	32
confounding	address confounding	68
confounding by indication	assess confounding by indication	69
contemporaneous record	maintain contemporaneous records	50
controlling version	identify the controlling version	23
corrective action	implement corrective action	50
cost-effectiveness	examine cost-effectiveness assumptions	69
coverage decision	track the coverage decision	68
cross-reference	verify the cross-reference	15

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

current good manufacturing practice	maintain CGMP compliance	50
data cutoff	state the data cutoff	33
data integrity	protect data integrity	50
data query	resolve a data query	24
dechallenge	record available dechallenge information	42
development candidate	nominate a development candidate	5
development milestone	plan the next development milestone	6
deviation investigation	conduct a deviation investigation	50
documentation gap	investigate a documentation gap	50
dose-response relationship	characterize the dose-response relationship	6
dossier	maintain a coherent dossier	15
duplicate report	investigate a possible duplicate report	42
effect size	interpret the effect size	33
effective date	verify the effective date	23
effectiveness check	plan an effectiveness check	51
electronic case report form	complete the electronic case report form	24
eligibility criterion	verify eligibility criteria	24
essential record	maintain essential records	24
estimand	specify the estimand	32
event onset	confirm event onset	42
evidence gap	prioritize evidence gaps	6
expectedness	evaluate expectedness	41
exploratory endpoint	label exploratory endpoints	32
exposure-response relationship	examine exposure-response relationships	6

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

external validity assess external validity	69
fair balance assess fair balance	59
final-form approval obtain final-form approval	60
focused question formulate a focused question	14
follow-up request issue a focused follow-up request	42
formulary review formulary status	68
go/no-go criterion set go/no-go criteria	6
good clinical practice apply good clinical practice	23
health technology assessment prepare for health technology assessment	68
hypothetical strategy justify a hypothetical strategy	32
indication define the proposed indication	15
indication boundary preserve the indication boundary	59
individual case safety report prepare an individual case safety report	41
information gap disclose an information gap	14
informed consent document the informed-consent process	23
institutional review board confirm institutional review board review	24
intended audience identify the intended audience	59
intercurrent event address intercurrent events	32
investigational new drug application prepare an IND submission	14
launch readiness verify launch readiness	69
lifecycle management align lifecycle management	69
market access develop the market-access plan	68
master production instruction follow master production instructions	50
material limitation retain material limitations	59

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

mechanism of action	investigate the mechanism of action	5
MedDRA	apply appropriate MedDRA coding	42
medical affairs	consult medical affairs	60
medical information	route a medical-information inquiry	60
medical-legal-regulatory review	complete MLR review	60
meeting minutes	reconcile meeting minutes	15
meeting package	assemble the meeting package	14
missing data	assess missing data	33
monitoring finding	document a monitoring finding	24
multiplicity	control multiplicity	33
net impression	evaluate the net impression	60
new drug application	plan the NDA content	14
nominal p-value	label a nominal p-value	33
nonclinical study	interpret nonclinical findings	6
observational study	describe the observational study	68
off-label use	distinguish an off-label question	60
onset claim	substantiate an onset claim	59
out-of-specification result	investigate an out-of-specification result	51
out-of-trend result	evaluate an out-of-trend result	51
p-value	interpret the p-value	32
payer	address payer evidence needs	68
pharmacodynamics	measure pharmacodynamic effects	6
pharmacokinetics	characterize pharmacokinetics	6
pharmacovigilance	support pharmacovigilance	41

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

point estimate present the point estimate	32
post hoc analysis label a post hoc analysis	33
prescribing information consult the prescribing information	59
preventive action define preventive action	51
primary endpoint report the primary endpoint	32
principal investigator consult the principal investigator	23
prior authorization clarify prior-authorization criteria	68
promotional claim substantiate a promotional claim	59
proof of concept specify the proof-of-concept question	5
protocol follow the applicable protocol	23
protocol amendment implement an authorized protocol amendment	23
protocol deviation assess a potential protocol deviation	24
qualifier keep the qualifier visible	60
randomization protect the randomization process	24
real-world data assess real-world data quality	68
real-world evidence evaluate real-world evidence	68
rechallenge record reported rechallenge information	42
redline review the redline	60
reference pack update the reference pack	60
reference product identify the reference product	69
reference safety information identify reference safety information	41
regulatory affairs consult regulatory affairs	15
regulatory commitment track regulatory commitments	15
regulatory pathway confirm the regulatory pathway	14

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

regulatory strategy	align the regulatory strategy	14
release authorization	verify release authorization	51
reporter	preserve reporter details	41
reproducibility	test reproducibility	6
residual confounding	acknowledge residual confounding	69
risk disclosure	improve risk disclosure	59
root cause	establish the root cause	50
safety signal	evaluate a safety signal	41
schedule of assessments	reconcile the schedule of assessments	23
scientific exchange	define the scientific-exchange context	60
secondary endpoint	interpret secondary endpoints	32
selection bias	examine selection bias	69
sensitivity analysis	perform sensitivity analysis	33
seriousness	assess seriousness	41
severity	record reported severity	41
significance threshold	apply the significance threshold	33
site activation	confirm site activation status	23
source record	preserve the source record	24
specification	confirm the applicable specification	51
sponsor position	state the sponsor position	14
stability program	maintain the stability program	51
statistical analysis plan	follow the statistical analysis plan	32
study coordinator	brief the study coordinator	23
subgroup analysis	qualify a subgroup analysis	33

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

submission readiness	assess submission readiness	15
supply constraint	communicate the supply constraint	51
supporting rationale	substantiate the supporting rationale	14
surrogate endpoint	justify a surrogate endpoint	5
target engagement	assess target engagement	5
target product profile	refine the target product profile	5
temporal association	distinguish temporal association from causation	42
testing hierarchy	respect the testing hierarchy	33
therapeutic window	investigate the therapeutic window	6
tolerability	evaluate tolerability	6
topline readout	prepare a balanced topline readout	33
traceability	preserve traceability	50
training record	verify the training record	24
translational evidence	strengthen translational evidence	5
treatment-policy strategy	define a treatment-policy strategy	32
unmet medical need	characterize the unmet medical need	5
unsolicited request	document an unsolicited request	60
uptake assumption	test the uptake assumption	69
value dossier	tailor the value dossier	68
verbatim term	retain the verbatim term	42
verification entry	complete the verification entry	50
version control	maintain version control	15
visit window	confirm the visit window	23
written response	review the written response	15

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] US Food and Drug Administration. The Drug Development Process.

[Open the original source](#)

Background on development stages and post-market monitoring. All products, figures, and conversations in this book are invented. Checked 30 September 2026.

[2] FDA / ICH. E9(R1): Estimands and Sensitivity Analysis in Clinical Trials.

[Open the original source](#)

Background for precise treatment-effect questions and interpretation. The fictional statistical exercise is not a trial-analysis specification. Checked 30 September 2026.

[3] FDA. Questions and Answers on CGMP Requirements: Records and Reports.

[Open the original source](#)

Background for quality documentation terminology. The batch dialogue does not prescribe release decisions or replace site procedures. Checked 30 September 2026.

[4] FDA. The Office of Prescription Drug Promotion.

[Open the original source](#)

Background for truthful, non-misleading promotion and review boundaries. Examples are invented, not approved product claims. Checked 30 September 2026.

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